



STRATEGIC INFORMATION

Reporting Month: April 2026

Reporting Period: March 2026

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HIV PREVENTION

Table 1 reflects the achievements of the Prevention Programme toward monthly and year-to-date (YTD) targets across various population groups:

Key Populations:

- For Men who have sex with men (MSM), reach was below the monthly target, achieving 74%, which brings the year-to-date target achievement to 23%. Testing also did not meet the monthly target, achieving 79%, thus achieving 24% of the annual target. Despite low coverage, the HIV positivity rate YTD was 1.7%.
- Transgender women (TG) did not meet the monthly target, achieving 73% for reach and 62% for testing. YTD performance towards the annual reach and test targets are 25% and 23%, respectively. TG had the highest YTD yield of 5% when compared to the other targeted populations.
- FSW performance also did not meet the monthly target, achieving 76% for both reach and test. YTD performance towards the annual reach and test targets are 29% and 30%, respectively. A relatively low YTD yield of 0.3%.

At Risk Youths (15-24 years):

- For ARY, reach was significantly below the monthly target, achieving 45%, this brings the year-to-date target achievement to 18%. Testing also did not meet the monthly target, achieving 46% also achieving 18% of the annual target. The HIV positivity rate YTD was 0.1%.

Sexually Active Females (25-49 years):

- Sexually active females (SAF) performed below target. Achievements were 49% for reach and 51% for test. YTD performance achievement towards the annual targets were 20% for reach and 21% for test, with an annual yield of 0.3%.

Sexually Active Males (25-49 years):

- Sexually active males (SAM) performed below the monthly targets for reach and test, with both achieving 40%. YTD performance stands at 17% for reach and 16% for test with a yield of 0.03%.

Adults (50+ years):

- Both older females and males did not meet their monthly targets, achieving 44% and 42% for both reach and 44% and 42% for test, respectively. The YTD performance for reach was 17% and 15%, and test 17% and 15%. The YTD for the target population yields for females was 0.05% and males 0.02%.

Table 1: HIV Reach & Test Achievements and Yields as of March 2026

Indicator	Monthly Target	Achievement (March)	% Achievement of Monthly Target	Annual Target (Jan-Dec 2025)	YTD Performance (Jan-March 2026)	% Achievement of YTD Target	YTD Comments
MSM reached	714	529	74%	8,573	1,955	23%	Monthly target met and annual target behind
MSM tested HIV	643	506	79%	7,716	1,883	24%	Monthly target met and annual target behind
Yield		15 (3%)		32(1.7%)			
TG reached	33	24	73%	390	98	25%	Monthly target met and annual target on track
TG tested HIV	29	18	62%	351	80	23%	Monthly target met and annual target behind
Yield		2 (11.1%)			4 (5 %)		
FSW reached	877	665	76%	10,522	3,069	29%	Monthly target met and annual target on track
FSW tested HIV	789	601	76%	9,470	2,809	30%	Monthly target met and annual target on track
Yield		3 (0.5%)		9 (0.3%)			
15-24 ARY reached	2,511	1,121	45%	30,134	5,519	18%	Monthly target met and annual target behind
15-24 ARY tested for HIV	2,260	1,043	46%	27,120	5,010	18%	Monthly target met and annual target behind
Yield		2 (0.2%)		5 (0.1%)			
25-49 SAF reached	4,098	2,027	49%	49,181	9,988	20%	Monthly target met and annual target behind
25-49 SAF tested HIV	3,689	1,899	51%	44,263	9,271	21%	Monthly target met and annual target behind
Yield		9 (0.5%)		24 (0.3%)			
25-49 SAM reached	2,861	1,148	40%	34,331	5,754	17%	Monthly target met and annual target behind
25-49 SAM tested HIV	2,575	1,042	40%	30,898	5,098	16%	Monthly target met and annual target behind
Yield		6 (0.6%)		15 (0.3%)			
50+ Females Reached	713	311	44%	8,550	1,452	17%	Monthly target met and annual target behind
50+ Females Tested	641	285	44%	7,695	1,327	17%	Monthly target met and annual target behind
Yield		0 (0%)		6 (0.5%)			
50+ Males Reached	721	306	42%	8652	1,322	15%	Monthly target met and annual target behind
50+ Males Tested	649	275	42%	7787	1,171	15%	Monthly target met and annual target behind
Yield		0 (0%)		2 (0.2%)			

Global Fund Prevention Indicators

The table below highlights the March 2026, reach and test performances towards the Global Fund Payment for results indicators. The monthly target achievements for reach and test were 92% and 109%, respectively. This resulted in 32% of the annual reach target and 38% of the annual test target being met. Linkages to PrEP services did not meet the monthly target, achieving 38%, bringing the achievement of the annual target to 18%. PrEP initiation achieved 45% of the monthly target and 19% of the annual target. OVP linked to treatment care and support services for the month were 3, achieving 33% of the monthly target and 6% of the annual target. OVP receiving other health services also did not meet the target, achieving 56% of the monthly target and 25% of the annual target. The distribution of HIV self-test kits was very low, achieving only 3% of the monthly target and 0.5% of the annual target.

Table 2: Global Fund Payment for Results Indicators as of March 2026

Indicator	Monthly Target	March	% Achievement of monthly Target	Global Fund Annual Target Jan- Dec 2026	Annual Target Performance Jan-March	% Achievement of Annual Target	Comments
Percentage of Other Vulnerable Populations Reached with HIV prevention programmes defined package of services	965	889	92%	11,580	3,657	32%	Monthly target not met
Percentage of Other Vulnerable Populations that have received an HIV Test and know their results	721	787	109%	8,652	3,298	38%	Monthly target exceeded
Number of Other Vulnerable Populations linked to PrEP services at least once during the reporting period.	60	23	38%	720	129	18%	Monthly target not met
Number of Other Vulnerable Populations who received a PrEP Product at least once during the reporting period (Initiated)	51	23	38%	616	118	19%	Monthly target not met
Number of Other Vulnerable Populations Linked to TCS	9	3	33%	105	6	6%	Monthly target not met
Number of Other Vulnerable Populations linked to other health services at least once during the reporting period.	54	30	56%	646	162	25%	Monthly target not met

Number of individual HIV Self-Test Kits Distributed	2,521	77	3%	30,248	155	0.5%	Monthly target not met
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The main risk behaviours identified in March were incorrect or inconsistent condom use (1,929), multiple sex partners (513), inconsistent use of contraceptive methods (372), type of sex/anal (170), substance misuse (92), History of STI (40). Gender based violence (35), Mental Health (16) and Type of sex (6) were the least behaviours identified. Condom distribution for risk behaviour identified was the highest among persons with incorrect or inconsistent condom use, with 87,192 distributed. Followed by persons with multiple partners as their risk behaviour at 23,607.

Table 3: Risk Behaviour Identified and Condom Distribution for March 2026

Risk Behaviour Identified	No. of Clients	Condoms Distributed (March)	YTD Condom Distributed
Incorrect or Inconsistent Condom Use	1,929	15,800	87,192
Multiple Sex Partners	513	4,625	23,607
Inconsistent Use of Contraceptive Methods	372	4,443	15,799
Type of Sex (Anal)	170	1,369	3,472
Substance Misuse	92	1,156	5,563
History of STIs	40	462	2,771
Gender Based Violence	35	393	4,745
Mental Health	16	171	3,839
Type of Sex (Orgies)	6	42	312

In March, the conversations around risk reduction predominantly focused on condom and lubricant use, with 6,070 discussions, closely followed by 6,062 conversations about HIV knowledge. Stigma and discrimination also played a significant role, generating 2,939 discussions. Other topics included partner reduction, family planning, and abstinence, which garnered 1,311, 910, and 917 discussions, respectively. On the other hand, issues like violence reduction and seeking care for STIs were less frequently discussed, with only 505 and 474 conversations taking place, while substance abuse and anal/genital health attracted even fewer discussions at 455 and 293.

Looking at the year-to-date figures, the pattern remains similar, with condom and lubricant use leading the charge at 28,955 discussions, and stigma and discrimination being a vital topic of concern for many.

Table 4: Risk Reduction Conversations for March 2026

Risk Reduction Conversations	No. of Interactions	YTD No. of Interactions
Condom and Lubricant use	6,070	28,955
Knowledge	6062	28,870
Stigma and Discrimination	2939	11,390
Partner Reduction	1311	6,646
Family Planning	910	3,426
Abstinence	917	3,557
Violence Reduction	505	2,466
Seeking Care for STIs	474	3,015
Substance Abuse	455	2,532
Anal / Genital care	293	2,314

Based on data obtained from DHIS2 for January to March 2026, a total of 24,257 persons tested negative for HIV. Of these, 14,711 were engaged with PrEP information, and 224 were documented as referred to access services. Subsequently, 409 were linked to PrEP services, and 160 were initiated in February. MSM had the largest initiation of 124, followed by FSW with 22, TG and SAF both with 5 and ARY and SAM both with 2. Inconsistency with data entry continues and has resulted in more persons being linked than referred (Table 5).

Table 5: PrEP Uptake for January to March 2026

Population	HIV Negative	Engaged	Referred	Linked	Initiated
SAF	9,301	5,793	23	198	5
SAM	5,102	3,249	9	55	2
ARY	5,027	3,642	11	2	2
FSW	2,839	1,421	29	23	22
MSM	1,904	568	145	126	124
TG	84	38	7	5	5
Total	24,257	14,711	224	409	160

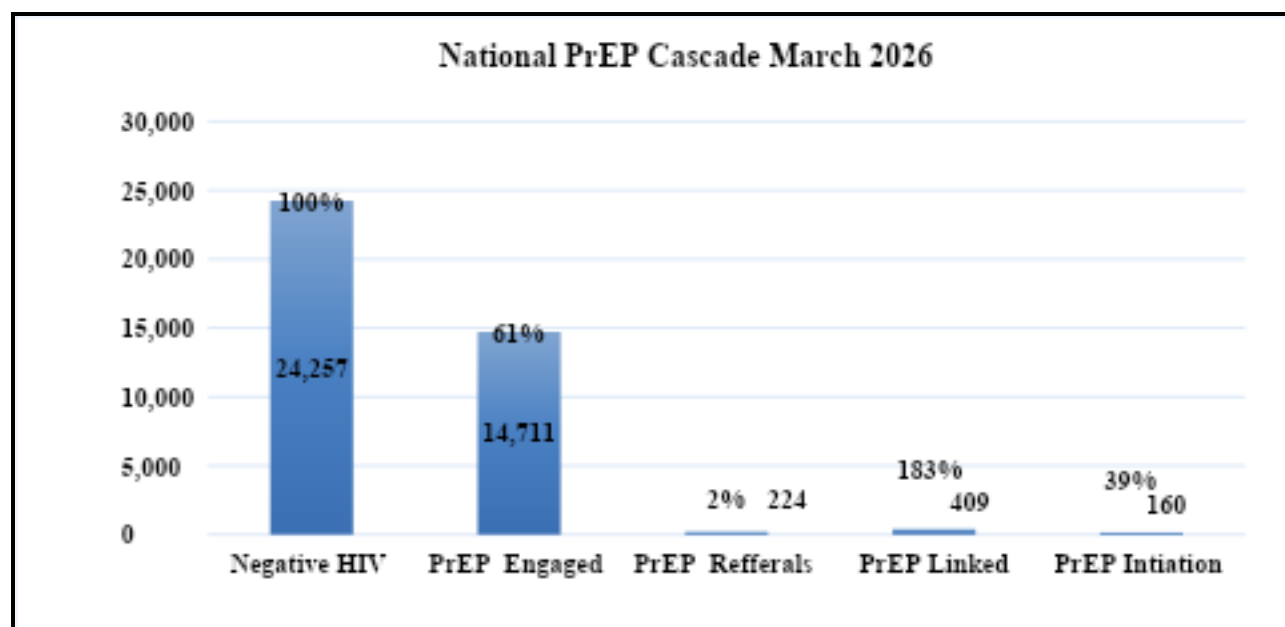


Figure 1: National PrEP Cascade as of March 2026

PrEP Cascade tracks stages in the PrEP process for HIV Prevention. It begins with 24,257 HIV-negative individuals and shows a progressive decline across subsequent stages: 14,711 (61%) were engaged, 224 (2%) were referred, 409 (183%) were linked, highlighting the data entry gap with the inconsistent entries for referrals. Subsequently, 160 (39%) were initiated on PrEP, indicating a substantial attrition rate for those who were linked (Figure 1).

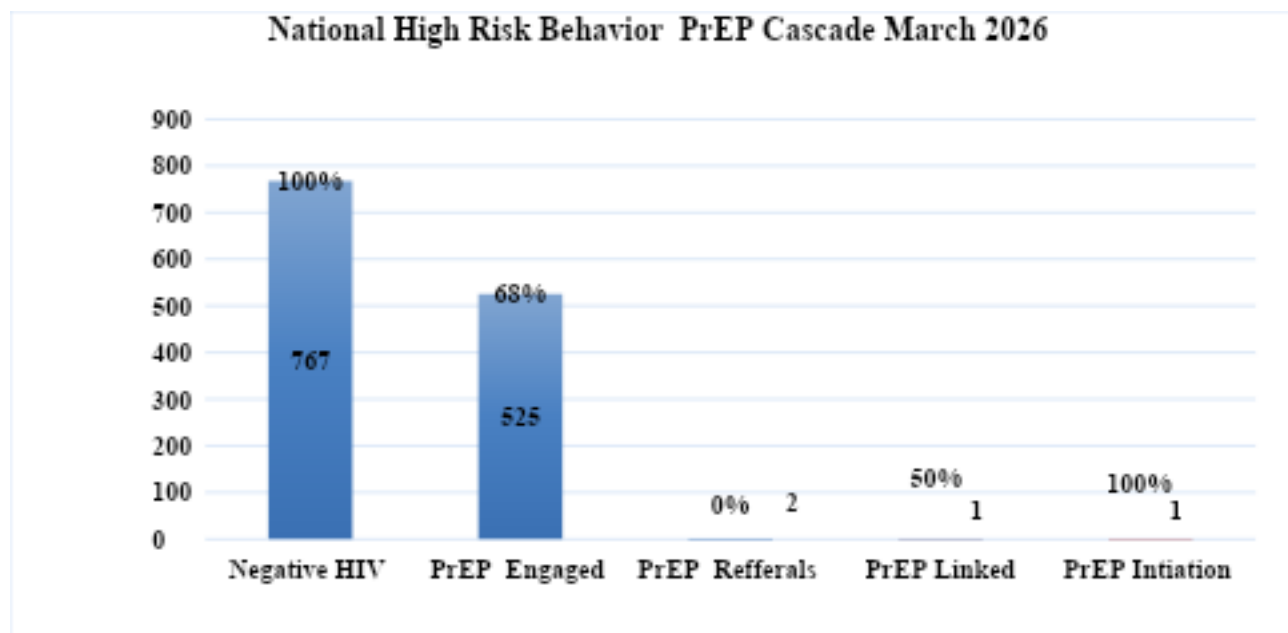


Figure 2: National High Risk Behaviour PrEP Cascade as of March 2026

PrEP Cascade for persons who reports high risk behaviours (History of STI and substance abuse) and tested negative for HIV. A total of 767 persons who reported high risk behaviour for the period January to March 2026 tested negative for HIV. 525 (68%) were engaged about PrEP of linked which a small fraction of 2 (less than 1%) were referred for PrEP services. Subsequently 1 person was linked representing 50%, resulting the one individual also initiated on PrEP representing 100%.

Self-Testing:

During the period from January to March 2026, a total of 1,340 HIV self-test kits were distributed. Of these, 1,257 were unassisted, and 83 were assisted. The distribution of kits among the genders was comparable; females accounted for most, 648 unassisted and 61 assisted, while distribution among males was 609 unassisted and 22 assisted. The largest distribution falls between the ages of 25 to 39 for females and males 20 and over. One reactive was recorded for a male who was assisted.

Table 6: HIV Self-Test distribution for January - March 2026						
Sex/Age Group	Assisted	Assisted Non-Reactive	Assisted Reactive	Unassisted	Unassisted Non-Reactive	Unassisted Reactive
Female	61	2	0	648	180	0
16 - 19	4	0	0	43	9	0
20 - 24	7	0	0	110	26	0
25 - 29	16	1	0	126	34	0
30 - 34	12	0	0	111	28	0
35 - 39	9	0	0	107	33	0
40 - 44	3	0	0	61	20	0
45 - 49	6	0	0	41	10	0
50+	4	1	0	49	20	0
Male	22	2	1	609	187	0
16 - 19	5	0	0	42	13	0
20 - 24	3	0	0	109	21	0
25 - 29	3	1	0	119	33	0
30 - 34	4	0	0	97	32	0
35 - 39	5	1	1	64	16	0
40 - 44	1	0	0	66	28	0
45 - 49	1	0	0	52	21	0

Table 6: HIV Self-Test distribution for January - March 2026						
Sex/Age Group	Assisted	Assisted Non-Reactive	Assisted Reactive	Unassisted	Unassisted Non-Reactive	Unassisted Reactive
50+	0	0	0	60	23	0
Total	83	4	1	1257	367	0

HIV Self Testing for Key Populations:

For key populations during the period from January to March 2026, a total of 127 HIV self-test kits were distributed. Of these, 112 were unassisted, and 15 were assisted. The distribution of kits was mainly to MSM (74) and FSW (51). Two kits were distributed to TG for the reporting period. No reactive result was reported.

Table 7: Self-Test Distribution for Key Population (Assisted and Unassisted) For January to March 2026						
Key Population	Assisted	Assisted Non-Reactive	Assisted Reactive	Unassisted	Unassisted Non-Reactive	Unassisted Reactive
FSW	15	0	0	36	3	0
MSM	0	0	0	74	3	0
TG	0	0	0	2	1	0
Total	15	0	0	112	7	0

National Quality Indicator Performances:

For the reporting period, HIV Prevention services reached 1,955 MSM, 3069 FSW and 98 TG. Of those, 4% of FSW, 4% of MSM and 7% of TG presented with high-risk behaviours. With respect to referrals, 6% of MSM, 3% of TG, and 1% of FSW received this service. The positivity yield from testing was 5% for TG, 2% for MSM, and 0.32% for FSW.

A total of 95% HIV self-test kits were distributed to MSM and 73% to FSW for primary use, while 50% were distributed to TG. Secondary use was 5% for MSM, and 27% for FSW, and 50% TG. In terms of self-tests given to high-risk individuals, 0% for all Key populations for primary use. Similarly, no self-tests were distributed for secondary use among any of the Key populations. Regarding PrEP initiation, 14% TG, 13% of MSM and 1% of FSW received PrEP.

Table 8: Quality Indicators for Key Populations as of March 2026

Indicators	Population	Achievement %
Number of self-test kits distributed for secondary use (to the OVP partner(s)) within the reporting period by vulnerable population (MSM, FSW, TG)	MSM	4% (78/1,955)
	FSW	4% (111/3069)
	TG	7% (7/98)
Percentage who received prevention basic services and were referred to other health services (e.g. psychological support, treatment for STIs or treatment care and support if HIV reactive)	MSM	6% (123/1955)
	FSW	1% (45/3,069)
	TG	3% (3/98)
Percentage of tested positive out of the total number tested for the reporting period (positivity yield)	MSM	2% (32/1883)
	FSW	0.32% (9/2809)
	TG	5% (4/80)
Percentage of self-test kits distributed for primary use (giving kits to the OVP member) within the reporting period	MSM	95% (95/100)
	FSW	73% (37/51)
	TG	50% (1/2)
Number of self-test kits distributed for secondary use (to the OVP partner(s)) within the reporting period	MSM	5% (5/100)
	FSW	27% (14/51)
	TG	50% (1/2)
Percentage of high-risk persons (History of STIs or Substance Misuse) receiving self-tests for primary use within the reporting period. Per population	MSM	0% (0/0)
	FSW	0% (0/0)
	TG	0% (0/0)
Percentage of high-risk (History of STIs or Substance Misuse) persons receiving self-tests for secondary use within the reporting period. Per population	MSM	0% (0/0)
	FSW	0% (0/0)
	TG	0% (0/0)

Indicators	Population	Achievement %
Percentage of high-risk (History of STIs or substance misuse) persons receiving any PrEP product during the reporting period and are registered in the DHIS2. Per population	MSM	13% (9/68)
	FSW	1% (1/109)
	TG	14% (1/7)
Percentage of high-risk persons (History of STIs or substance misuse) linked to PrEP. Per population	MSM	15% (10/68)
	FSW	1% (1/109)
	TG	14% (1/7)

PREVENTION OF MOTHER TO CHILD TRANSMISSION

A review of the database revealed that just over half of the expected reports were submitted. Of the 39 reports expected, 28 were received, representing a 72% submission rate. The Western Region demonstrated the strongest performance, submitting 11 of 12 expected reports (92%), all of which were on time. The Northeast Region followed with 78% completeness (7 of 9 reports), also maintaining full timeliness among submitted reports.

In contrast, the Southern Region showed lower performance, with less than half of the expected reports submitted (44%). Notably, the Southeast Region recorded 6 submissions, resulting in 66% completeness and timeliness. However, these reports were not reported as being on time as they failed to click complete on the database.

Table 9: Reporting Compliance for PMTCT reports January-March 2026

Region	Expected Reports	Actual Reports	Percent Received	Reports On Time	Percent On Time
Western Region	12	11	92	92	92
Northeast Region	9	7	78	78	78
Southern Region	9	4	44	44	44
Southeast Region	9	6	66	0	0
Total	39	28	72	56	56

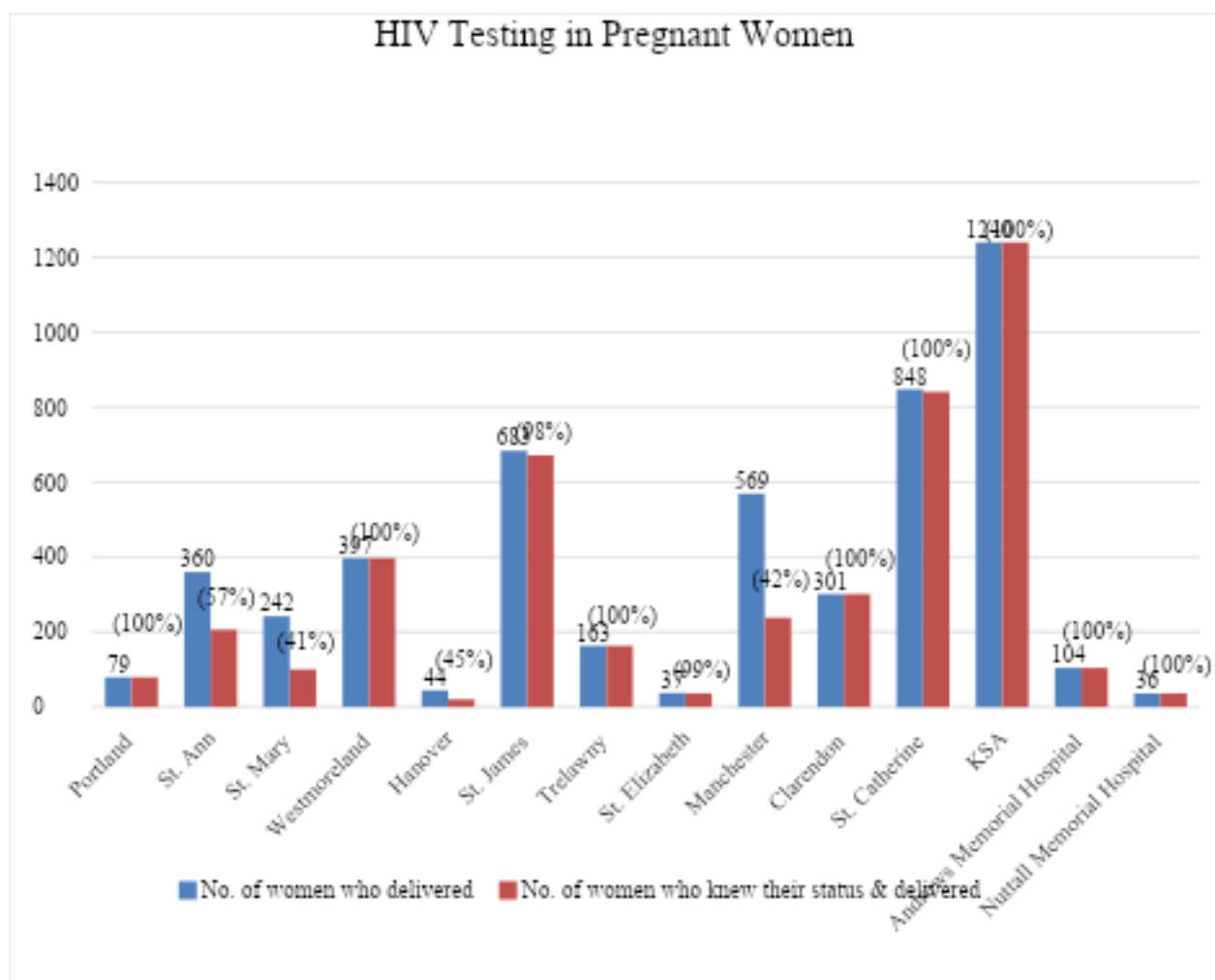


Figure 3: HIV testing in pregnant women who delivered

A review of the country's PMTCT data for the reporting period January-March indicates that HIV testing through antenatal care (ANC) services was offered to 4,432 of the 5,103 pregnant women who delivered, resulting in an HIV testing coverage of 87% (Figure 3). Notably, no reports were submitted from St. Thomas for the reporting period. Several parishes and private hospitals achieved full (100%) status awareness at delivery. Notably, Portland, Westmoreland, Trelawny, Clarendon, KSA, Andrews Memorial Hospital, and Nuttall Memorial Hospital all recorded 100% of women knowing their HIV status at the time of delivery. Whereas St. James (98%) and St. Elizabeth (99%) performed well but still fell slightly short of full coverage. However, some parishes demonstrated gaps in coverage. St. Ann (57%), St. Mary (41%), Hanover (45%), and Manchester (42%) had notably lower proportions of women aware of their HIV status despite moderate to high numbers of deliveries.

From January to March 2026, a total of 61 HIV-positive women delivered. Of these, 55 received antiretroviral therapy (ART) $\geq$ 4 weeks prior to delivery, while 6 received ART $<$ 4 weeks prior to delivery. During the same period, 61 HIV-exposed infants were born and all 61 infants received antiretroviral prophylaxis within 72 hours of birth to reduce the risk of mother-to-child transmission (MTCT) (Figure 4).

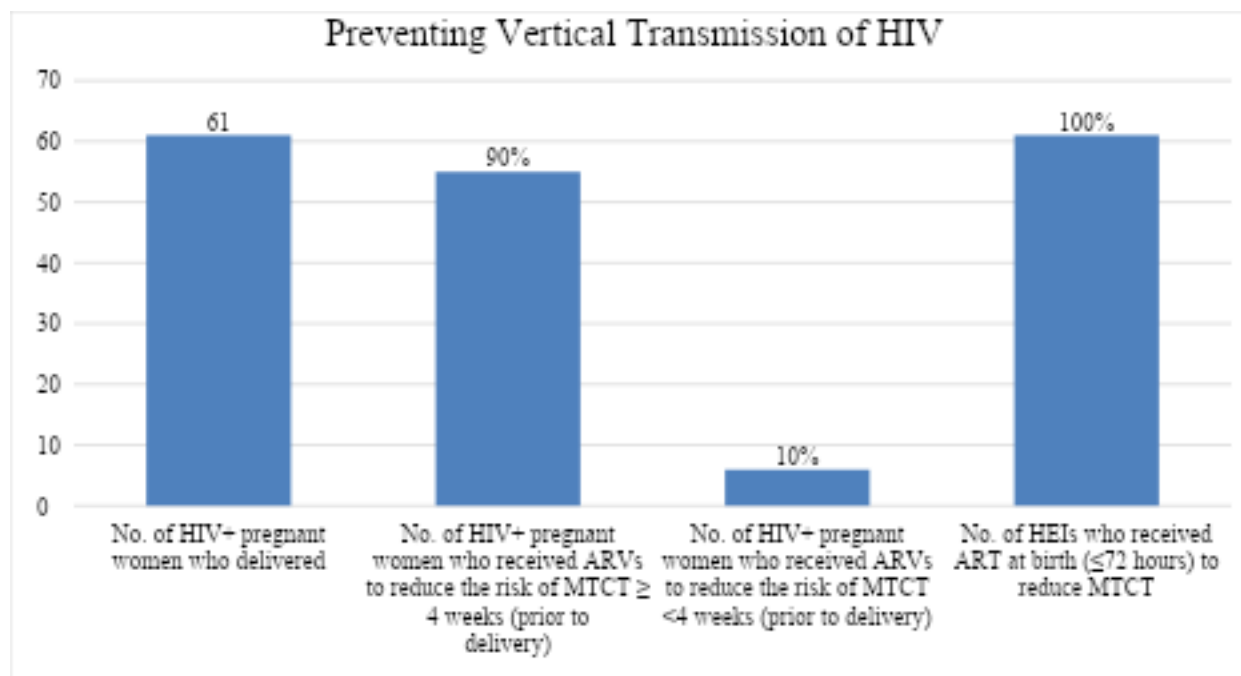


Figure 4: Preventing vertical transmission of HIV

A review of the reporting period indicates a syphilis testing coverage of 92%, with a positivity rate of 1% (40 cases). Among the 40 women who tested positive for syphilis and delivered during the period, no cases of congenital syphilis were reported. Of these women, 31 received at least one dose of Benzathine Penicillin 2.4 million units intramuscularly, while 9 did not receive treatment, resulting in a treatment coverage of 78% for the reporting period (Figure 5).

The presence of 9 untreated syphilis-positive mothers alongside the reporting of zero congenital syphilis cases is a cause for concern. According to the WHO case definition for congenital syphilis, these 9 untreated pregnancies would be classified as congenital syphilis cases. Strengthening timely diagnosis and treatment of syphilis-positive pregnant women remains critical to the elimination of congenital syphilis.

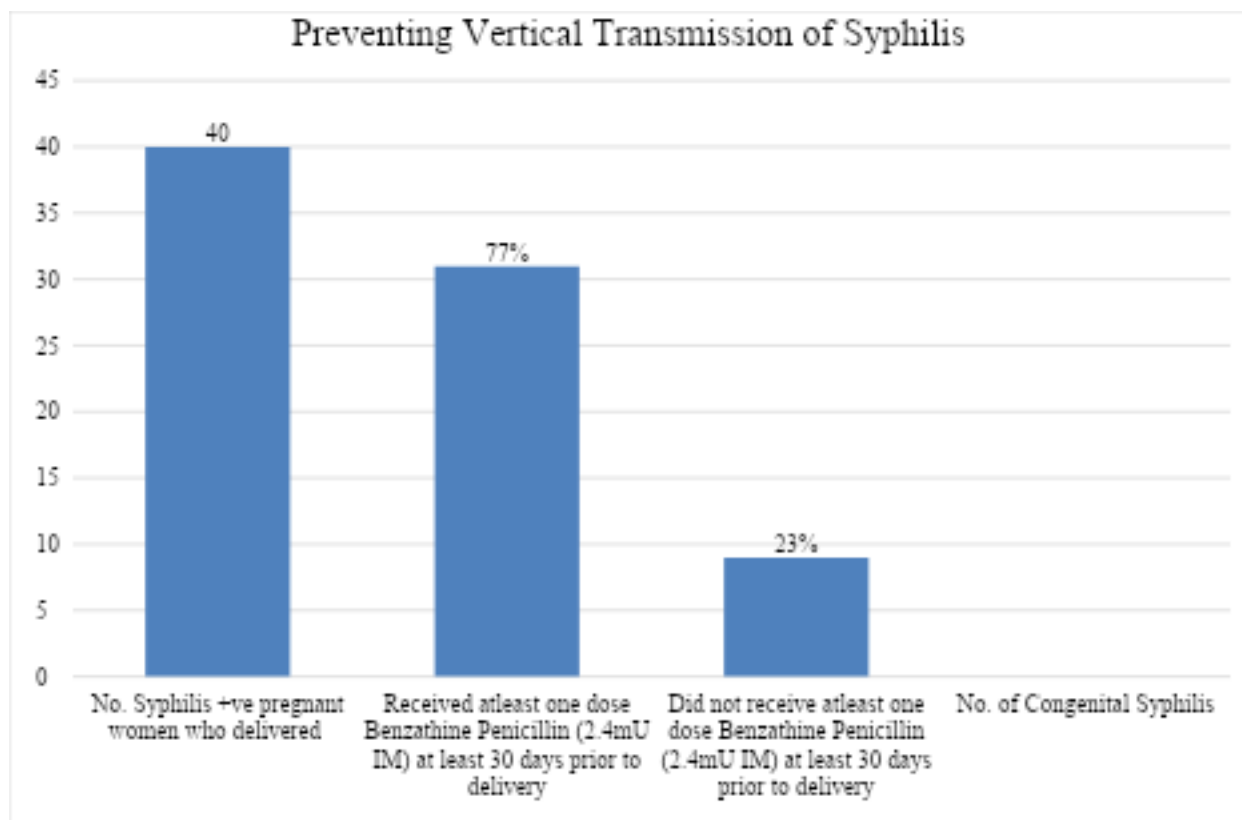


Figure 5: Preventing vertical transmission of Syphilis

HIV TREATMENT

HIV Surveillance

In March 2026, **60** cases of HIV were confirmed in TSIS, with a YTD total of 185. When comparing performance for the same period in 2025, 206 cases were confirmed, which is 21 fewer cases than in 2026. Please see the data below.

Table 10: Year-to-date (YTD) comparison of confirmed cases for 2026 and 2025

Month	Cases 2026	Cases 2025
January	58	84
February	67	54
March	60	68
Total YTD	185	206

Private Lab Monitoring

The SI Unit re-engaged major private laboratories to provide monthly HIV reports. During the reporting period, only two laboratories (Consolidated and Sagicor) submitted their reports on time. A total of 443 HIV tests were conducted, with 32% (n=143) of the cases being males and 68% (n=300) females. Notably, there were no positive cases reported. The SI unit, through our monthly engagements, continues to remind these private labs that the submission of the class one notification form to the national surveillance unit is mandatory once a case is detected.

Table 11: HIV Monthly Reports from Private Labs

Laboratory	Report Received:	No. Tested		Negative Results		Positive Results		Inconclusive Results	
		No. of males	No. of females	No. of males	No. of females	No. of males	No. of females	No. of males	No. of females
Sagicor	Yes	103	167	103	167	0	0	0	0
Guardian Life	No	-	-	-	-	-	-	-	-
Andrew's Memorial Hospital	No	-	-	-	-	-	-	-	-
Central Medical	No	-	-	-	-	-	-	-	-
Biomedical	No	-	-	-	-	-	-	-	-
Hargreaves	No	-	-	-	-	-	-	-	-
Microlabs Limited	No	-	-	-	-	-	-	-	-
Medical Associates	No	-	-	-	-	-	-	-	-
Eagles Medical	No	-	-	-	-	-	-	-	-
Consolidated	Yes	40	133	40	133	0	0	0	0
Total		143	300	143	300	0	0	0	0

National Treatment Cascade (Quarterly)

Based on the UNAIDS Spectrum estimation process for 2025, the estimate for persons living with HIV (PLHIV) is **28,000**, of which **26,500 (95%)** are estimated to have been diagnosed.

As of the end of **March 2026**, **19,346 (69%)** PLHIV were linked to care nationally. Additionally, **15,695 (56%)** PLHIV have been retained in care, and **14,954 (53%)** PLHIV are retained on ARV. **12,360 (44%)** had a valid viral load test, and **11,302 (40%)** were virally suppressed (Figure 6).

Over the past quarter, as represented in Figure 6, all indicators' percentages remained the same.

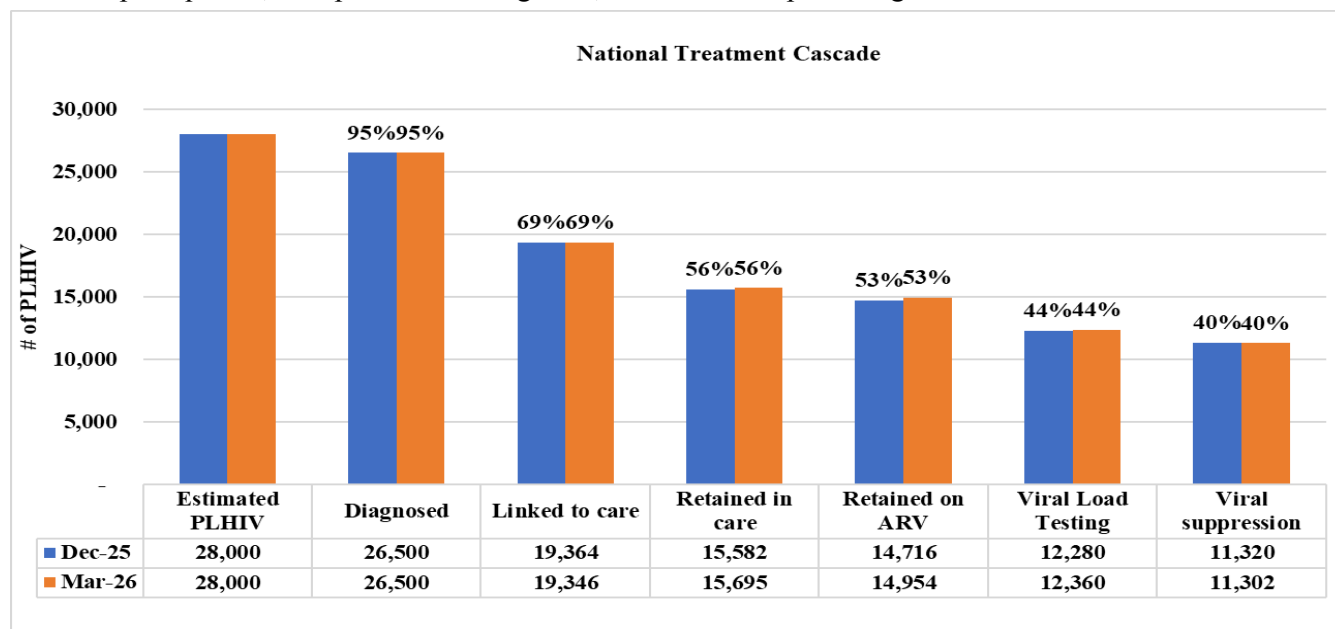


Figure 6: National Treatment Cascade as of March 2026

All RHA sites, private clinicians, and independent sites are represented in the cascade; it shows that of the **26,500** PLHIV who know their status, **7,154 (27%)** have not yet been linked to care. Furthermore, of the **19,346** PLHIV linked to care, **3,651 (19%)** are lost to follow-up.

National 95-95-95 Cascade

The national 95-95-95 cascade indicates that the first gap has been achieved, a **39%** gap for the second 95, and a **19%** gap for the third 95 (Figure 7).

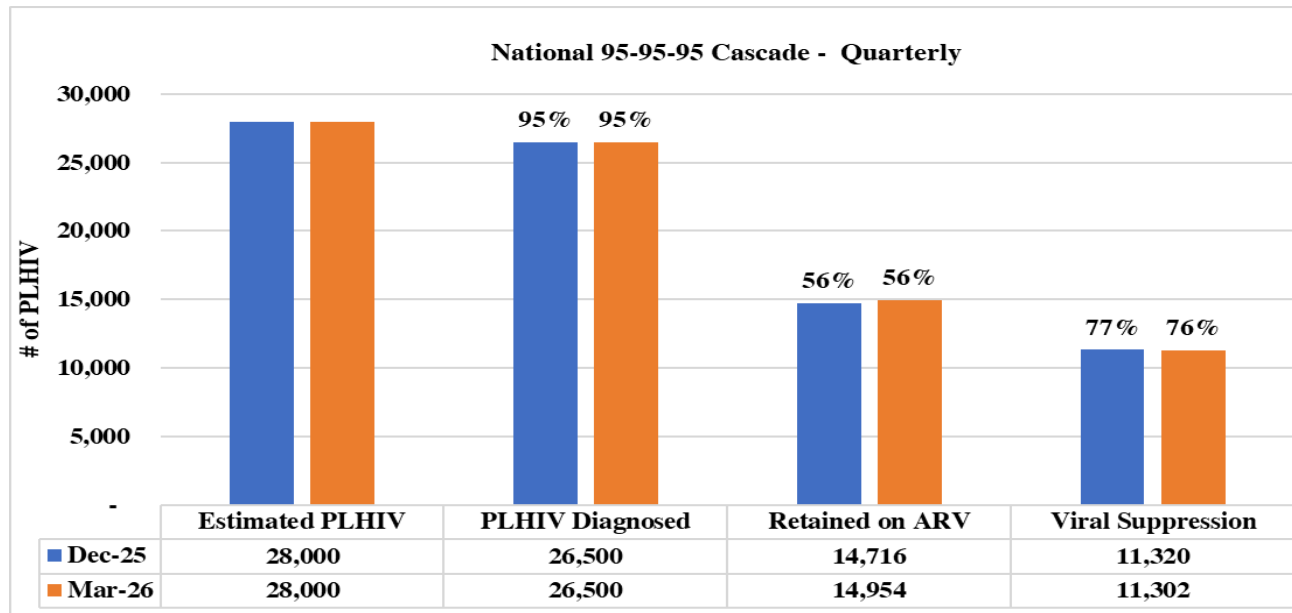


Figure 7: National 95-95-95 Cascade as of March 2026

Monitoring and Evaluation

Target: 95%

Met/surpassed national target ($\geq 95\%$)
Moderate performance (70% - 94%)
Underperformance (50% - 69%)
Critical Underperformance ($\leq 50\%$)

Table 12: Specific Cascade Focus Data Disaggregated by Sex and Age as of March 2026

Age	Linked to Care		Retained in Care		Retained on ARV		Viral Load Testing		Viral Suppression	
	Male	Female	Male	Female	Male	Female	Male	Female	Male	Female
0-4	7	14	100%	100%	86%	100%	71%	86%	43%	86%
5-9	18	34	89%	97%	83%	97%	61%	74%	56%	62%
10-14	31	31	94%	100%	87%	100%	74%	87%	48%	68%
15-19	69	107	93%	87%	88%	79%	64%	64%	46%	53%
20-24	290	445	82%	75%	76%	69%	58%	53%	52%	44%
25-29	605	789	77%	71%	73%	66%	61%	51%	57%	45%
30-34	848	1,124	74%	73%	70%	69%	56%	57%	52%	50%
35-39	851	1,315	73%	77%	67%	72%	53%	59%	48%	55%
40-44	918	1,342	78%	80%	74%	75%	59%	62%	54%	57%
45-49	995	1,338	79%	84%	74%	80%	62%	68%	56%	63%
50-54	1,180	1,240	80%	89%	77%	86%	65%	72%	60%	66%
55-59	1,112	1,010	83%	86%	80%	84%	66%	72%	60%	67%
60-64	840	816	88%	89%	85%	86%	72%	72%	66%	68%
65-69	524	525	88%	88%	85%	85%	71%	73%	66%	68%
70+	471	453	86%	87%	81%	85%	70%	73%	65%	68%
Total	8,762	10,584	80%	82%	76%	78%	63%	65%	57%	59%

Table 12 above highlights the national achievements in attaining 95% or more of PLHIV linked to care to be retained in care, retained on ARV, receive viral load testing and achieve viral suppression. For the retained in care column, PLHIV, both males and females 0-4, females 5-9 and 10-14 surpassed the target. For the retained on ARV column, females 0-4, 5-9 and 10-14 surpassed the target. Viral load testing and viral suppression require major improvement to meet the 95% target.

Newly Linked and Newly Initiated ARV (Quarterly)

Tables 13 and 14 below indicate linkage and ARV initiation for the past 2 quarters. The data shows an increase in linkage to care during January - March 2026 compared to October - December 2025. A similar trend is seen for those newly initiated on ART.

Table 13: PLHIV Newly Linked to Care

	Newly Linked to Care		
	October - December 2025	January - March 2026	Total
Male	80	109	189
Female	85	108	193
Total	165	217	382

Table 14: PLHIV Newly Initiated on ART

	Newly Initiated on ART		
	October - December 2025	January - March 2026	Total
Male	77 (*79)	104 (*105)	181 (*184)
Female	84 (*94)	104 (*115)	188 (*209)
Total	161 (*173)	208 (*220)	369 (*393)

**includes patients linked from the previous months.*

Figure 8 below highlights a higher proportion of missing CD4 results during the January - March 2026 quarter compared to the October - December 2025 quarter. Of those, with a CD4 result, a CD4 >350 remains to have the highest proportion.

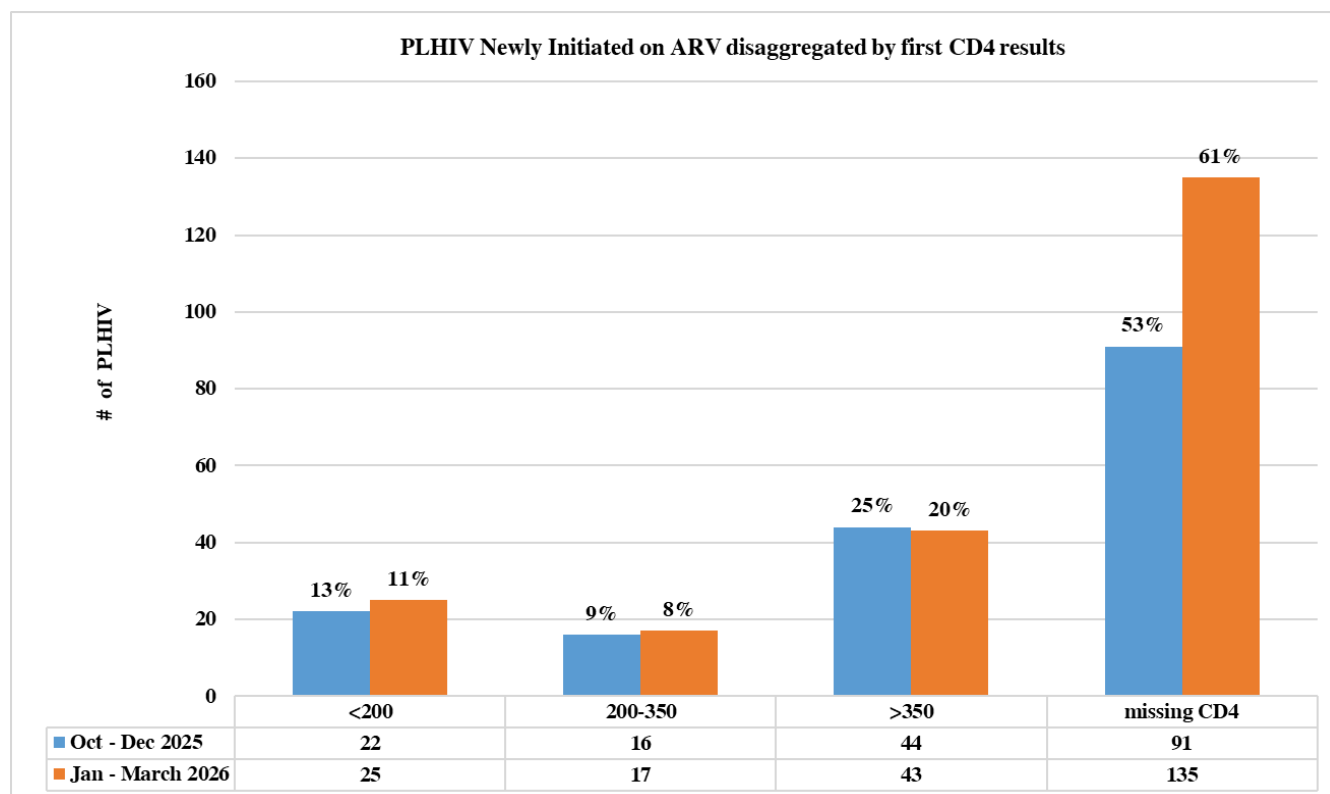


Figure 8: Newly Initiated on ART per quarter disaggregated by first CD4 results

Table 15 below highlights a higher proportion of females having a missing CD4, with the majority being within the 30-34 age group. Of those with a CD4 <200, males and females were nearly balanced. For the males, the majority with a CD4 <200 were within the 30-49 age group, while for the females, the age groups were staggered.

Table 15: First CD4 result for PLHIV newly initiated on ART disaggregated by sex and age group

Age	Missing CD4		<200		200-350		>350	
	Male	Female	Male	Female	Male	Female	Male	Female
0-4	0	1	0	0	0	0	1	0
5-9	0	0	0	0	0	0	1	0
10-14	0	1	0	1	0	1	0	0
15-19	5	6	0	0	0	0	0	0
20-24	5	8	0	0	2	0	6	5
25-29	11	6	0	2	0	0	0	3
30-34	9	12	3	1	0	2	1	5
35-39	5	9	3	1	1	1	2	4
40-44	7	6	2	1	2	1	0	3
45-49	5	6	3	0	0	0	1	3
50-54	6	6	1	2	2	1	0	1
55-59	5	2	0	1	0	1	1	1
60-64	1	2	0	2	2	1	1	1
65-69	2	2	0	0	0	0	2	0
70+	4	3	2	0	0	0	1	0
Total	65	70	14	11	9	8	17	26
%	30%	32%	6%	5%	4%	4%	8%	12%

Viral Suppression within 6 months of initiating ARV

The graph below shows the viral suppression of those eligible for viral load testing initiated (6 or more months after first ARV) at the end of December 2025 and March 2026. Viral load eligibility increased by 1%, while viral load testing and viral suppression declined by 1%.

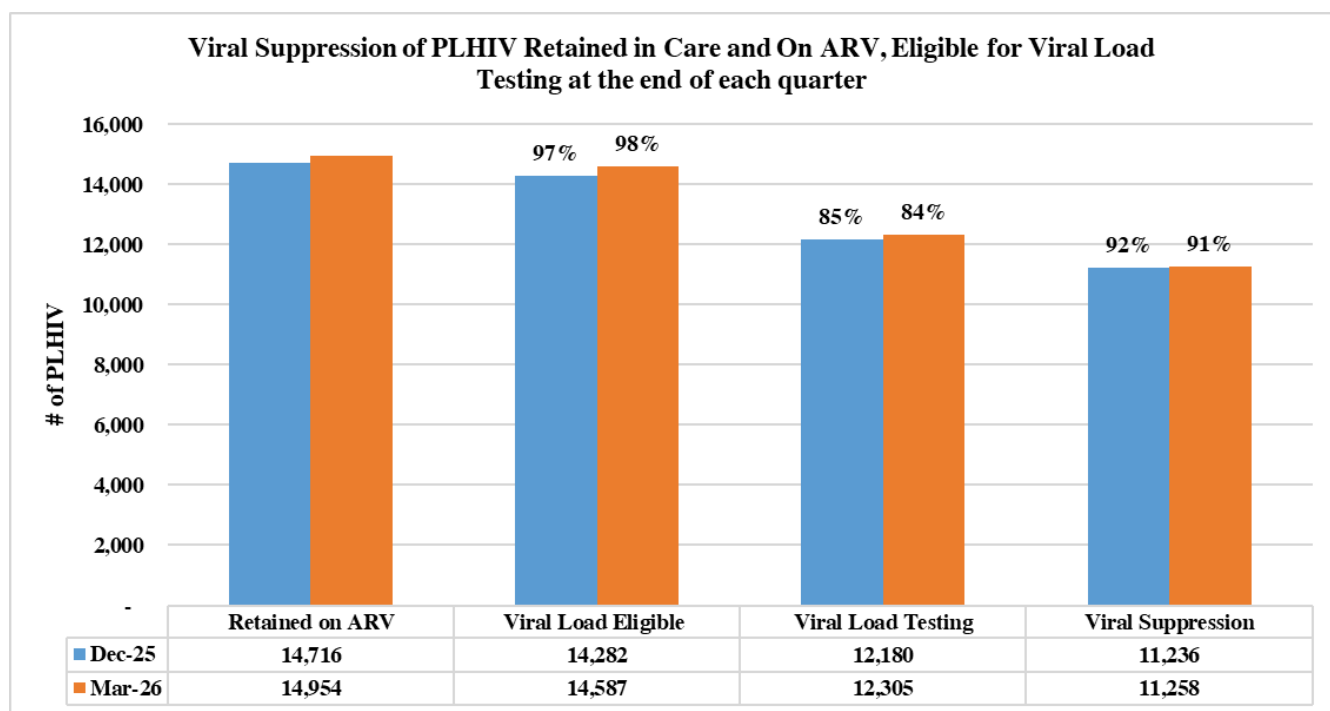


Figure 9: Viral Suppression within 6 or more months of initiating ARV

Lost to follow-up (Quarterly)

The figure below indicates that the lost to follow-up percentages have remained the same over the past 2 quarters. A data verification process was undertaken using NSUs death data to verify if some of the PLHIV lost to follow-up have died. These updates are currently being made in TSIS, and the effect on the lost to follow-up numbers should be reflected in the April 2026 report.

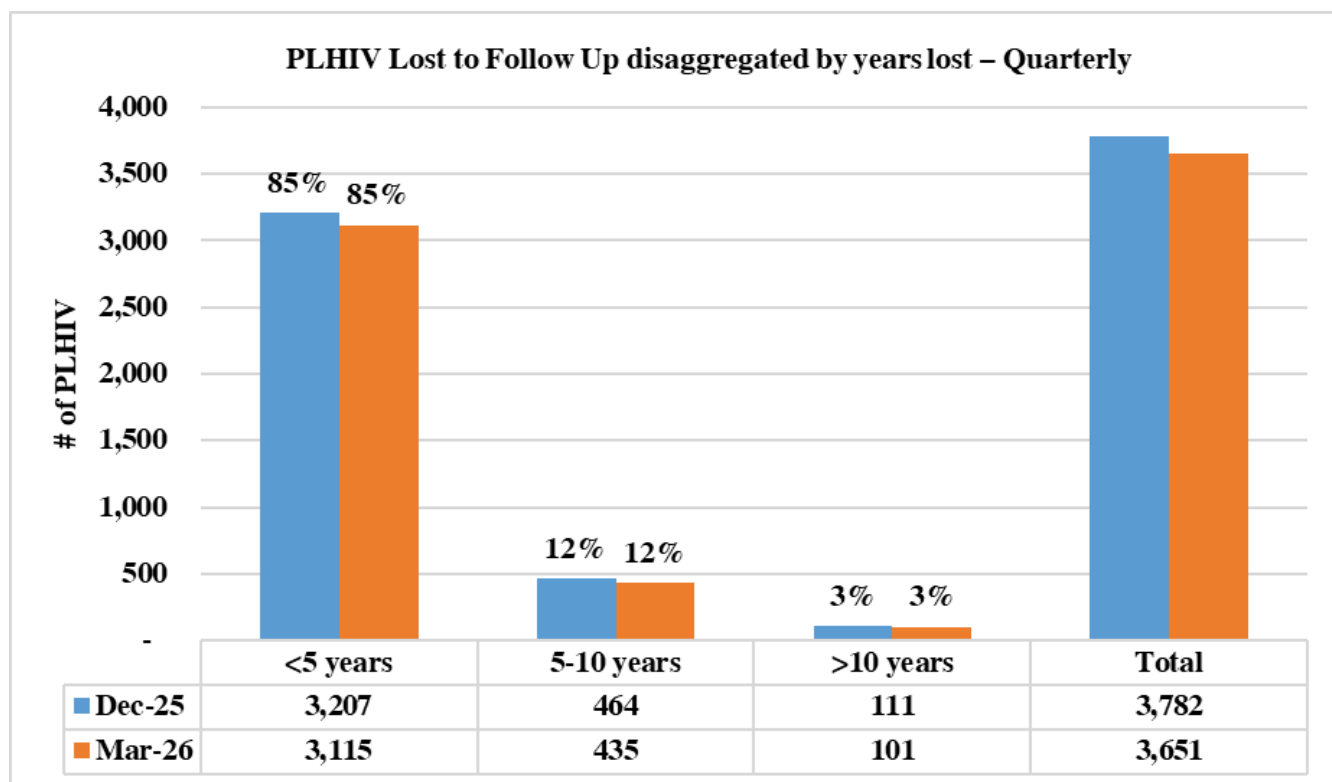


Figure 10: Total LTFU disaggregated by years lost - Quarterly

The figure below shows an examination of the PLHIV retained in care but not currently on ART at the end of 3 quarters from September 2025 to March 2026. The total number of PLHIV retained in care but not retained on ARV declined throughout the 3 quarters, with the lowest number recorded seen at the end of March 2026. These data quality issues will continue to be addressed in the TSIS data quality meetings (Figure 11).

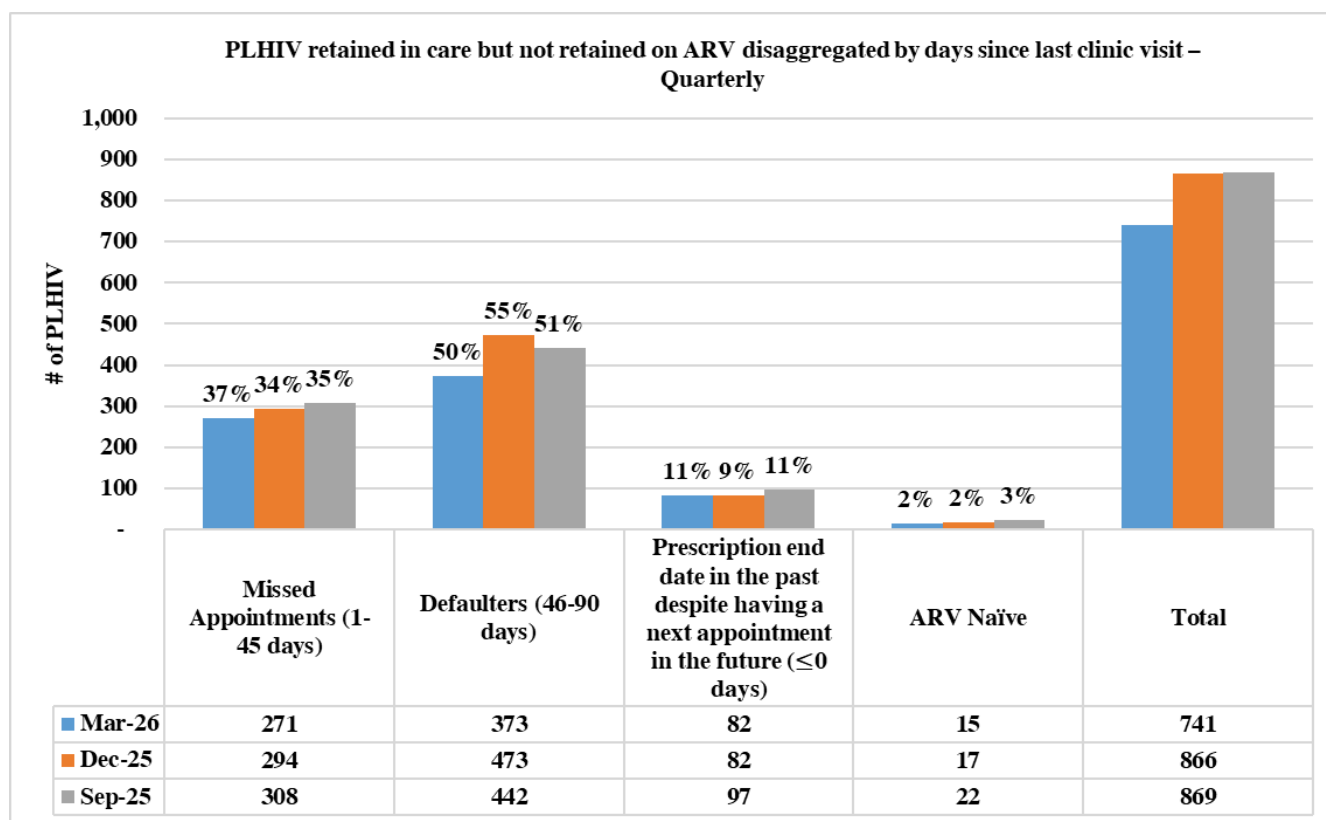


Figure 11: PLHIV retained in care but not retained on ARV

Table 16 displays the Global Fund Treatment, Care and Support targets for 2026-2027. As of March 2026, the target for children on ART was surpassed on a national level.

Table 16: Global Fund Treatment, Care and Support Indicators as of March 2026

Indicator	Annual Target	Performance	Achievement
% of People on ART among all people living with	73.83%	14,954/28,000	53%

HIV at the end of the reporting period.			
^a % of Children (under 15) on ART among all children living with HIV at the end of the reporting period.	84.18%	130/135	96%
^b % of People newly diagnosed with HIV initiated on ART.	97%	176/185	95%
^c % of People living with HIV and on ART with viral load test results.	95%	12,305/14,587	84%

^a All children living with HIV are estimated from TSIS.

^b HIV confirmed date used as a proxy for newly diagnosed PLHIV for 2026.

^c People living with HIV on ART who are eligible for a viral load test.

Regional Authorities Cascades

Quarterly

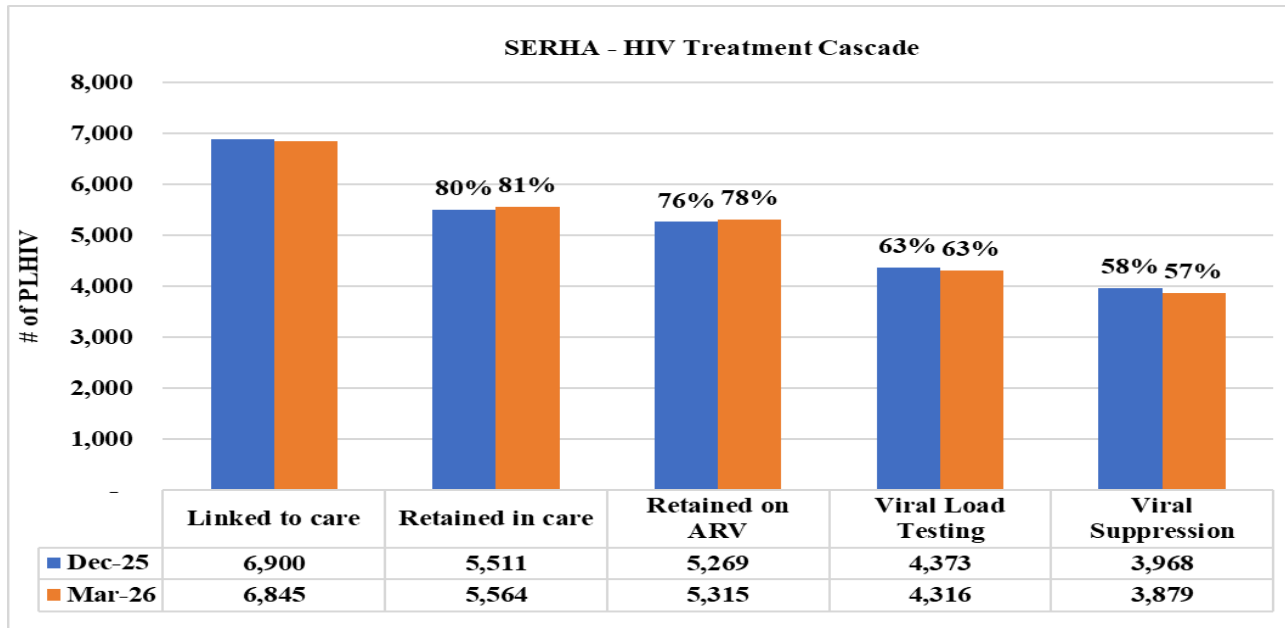


Figure 12: SERHA - HIV Treatment Cascade

March 2026, when compared to December 2025, retained in care increased by 1%, retained on ARV increased by 2%, viral load testing remained the same, and viral suppression declined by 1%. SERHA continues to experience challenges with viral load result processing turnaround times. Sites have been engaging with NPHL directly to retrieve results in an effort to mitigate further impact on the treatment cascade (Figure 12).

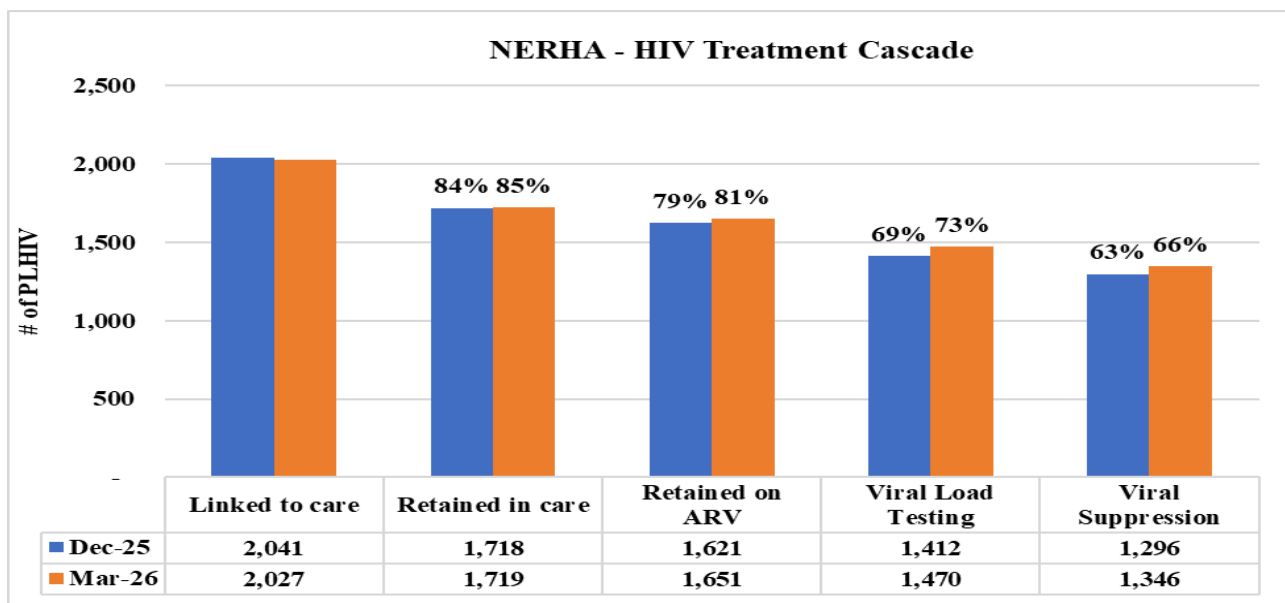


Figure 13: NERHA - HIV Treatment Cascade

March 2026, when compared to December 2025, retained in care increased by 1%, retained on ARV increased by 2%, viral load testing increased by 4%, and viral suppression increased by 3%. NERHA continues to experience challenges with viral load result processing turnaround times. Sites have been engaging with NPHL directly to retrieve results in an effort to mitigate further impact on the treatment cascade. A line list was sent to TCS to investigate long-awaited viral load results (Figure 13).

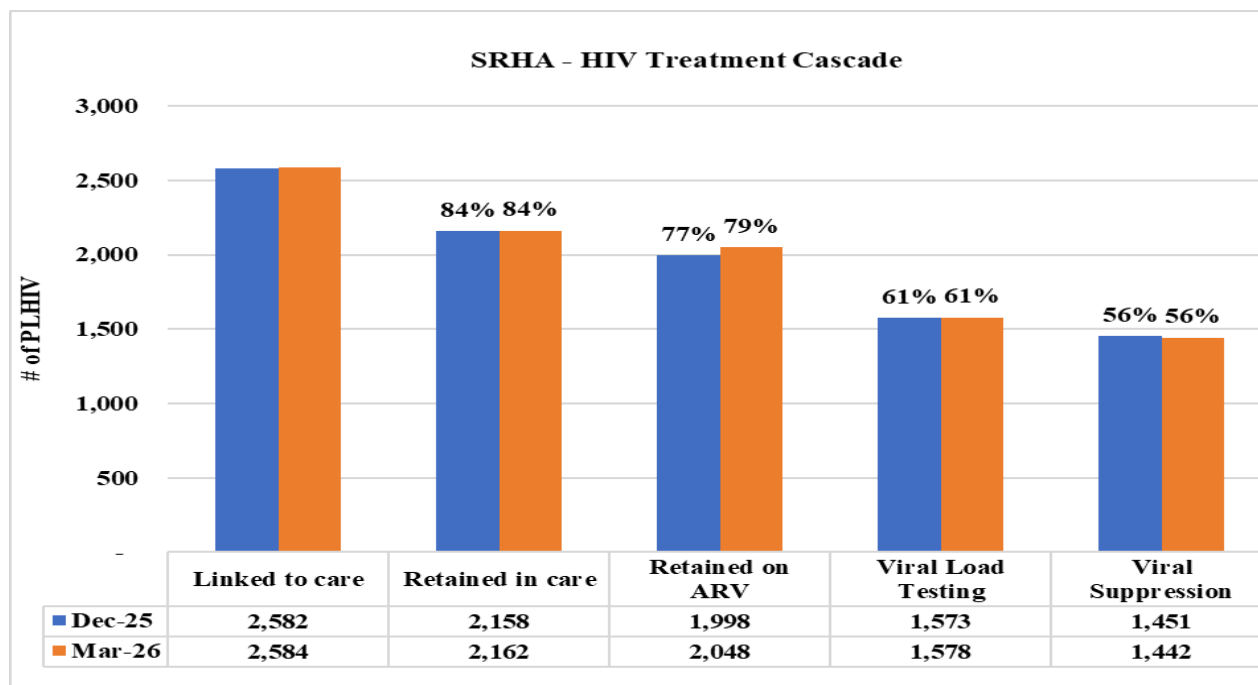


Figure 14: SRHA - HIV Treatment Cascade

March 2026, when compared to December 2025, retained in care remained the same, retained on ARV increased by 2%, viral load testing and viral suppression remained the same. SRHA continues to experience challenges with viral load result processing turnaround times. Sites have been engaging with NPHL directly to retrieve results in an effort to mitigate further impact on the treatment cascade (Figure 14).

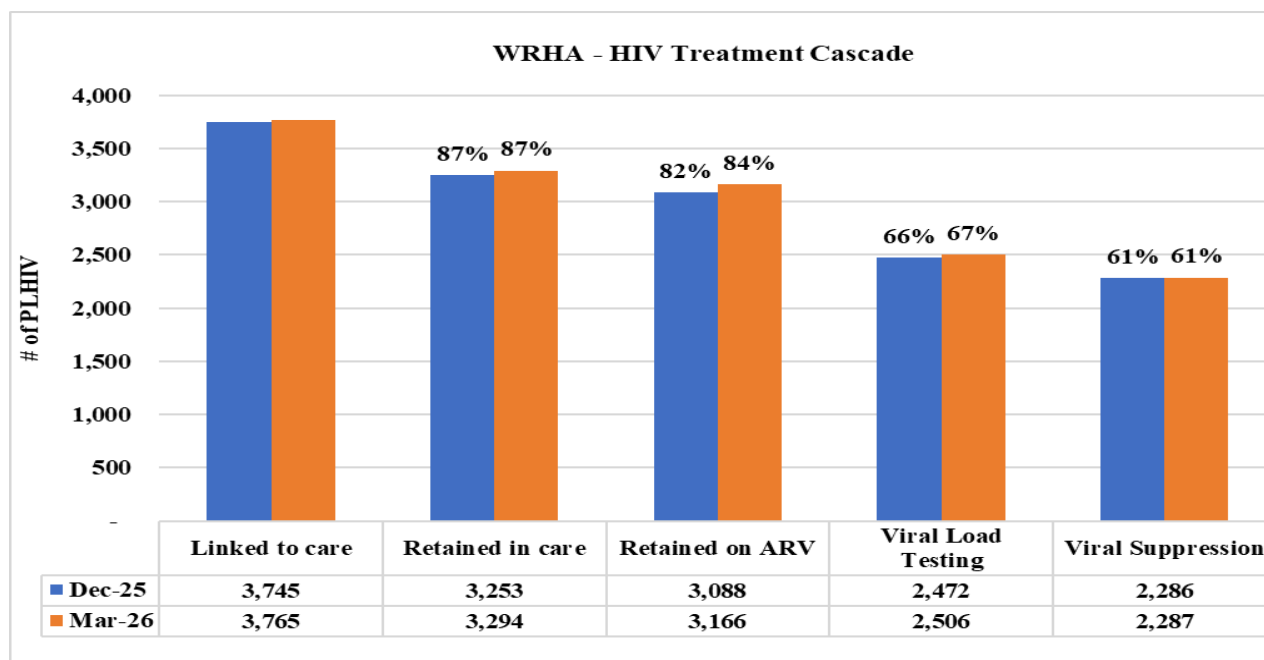


Figure 15: WRHA - HIV Treatment Cascade

March 2026, when compared to December 2025, retained in care remained the same, retained on ARV increased by 2%, viral load testing increased by 1%, and viral suppression remained the same. WRHA continues to experience challenges with viral load result processing turnaround times. Sites have been engaging with NPHL directly to retrieve results in an effort to mitigate further impact on the treatment cascade (Figure 15).

Independent Sites Cascades

Quarterly

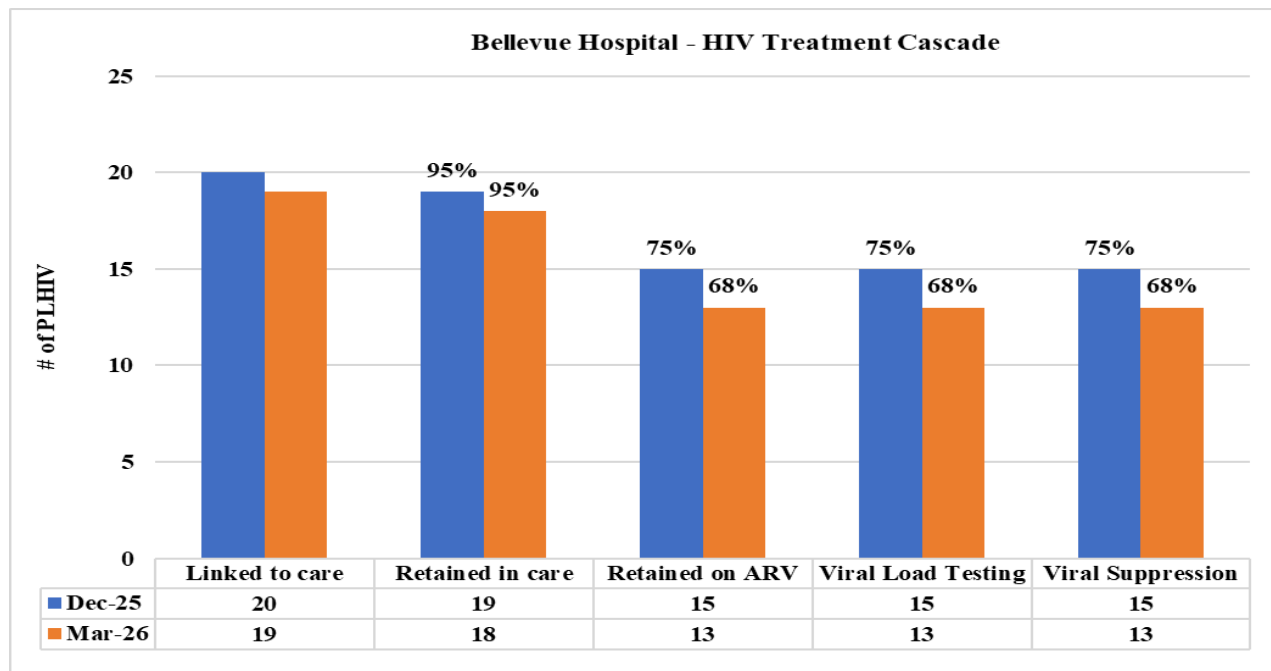


Figure 16: Bellevue - HIV Treatment Cascade

As of March 2026, compared to December 2025, retained in care remained the same. However, retention on ARV, viral load testing and viral suppression declined by 7%. Bellevue continues to experience challenges with data management. SI will be meeting to think of a strategy to assist the Bellevue team (Figure 16).

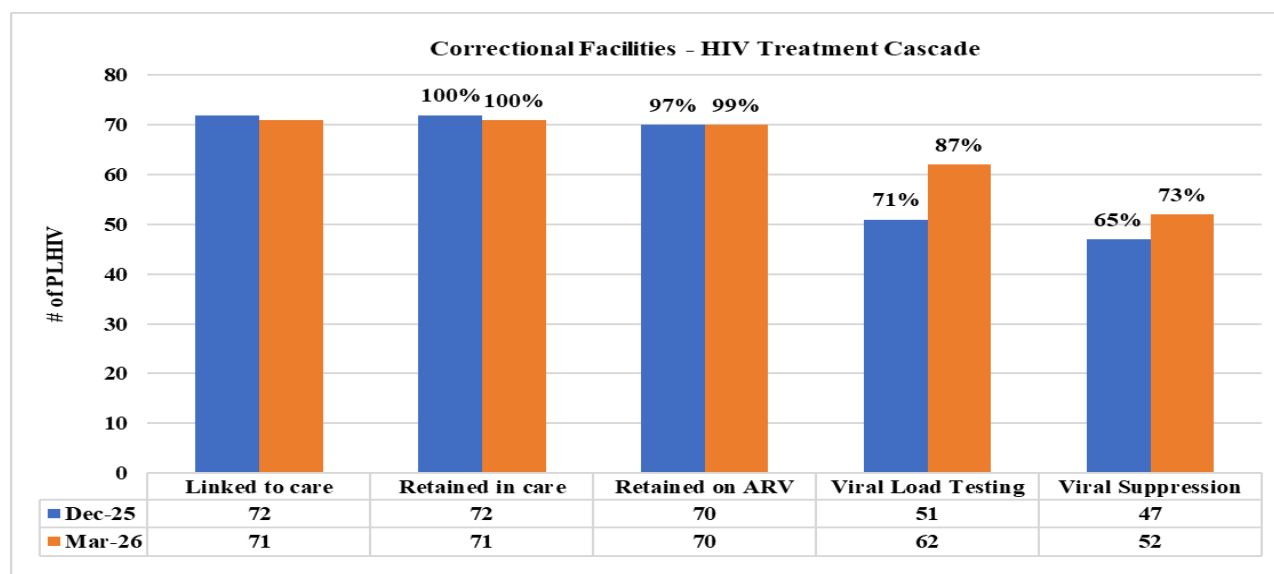


Figure 17: Correctional Facilities - HIV Treatment Cascade

March 2026, when compared to December 2025, retained in care remained the same, retained on ARV increased by 2%, viral load testing increased by 16%, and viral suppression increased by 8%. Improvements in the viral load result and viral suppression pillars are attributed to revised data entry practices for viral load results in TSIS (Figure 17).

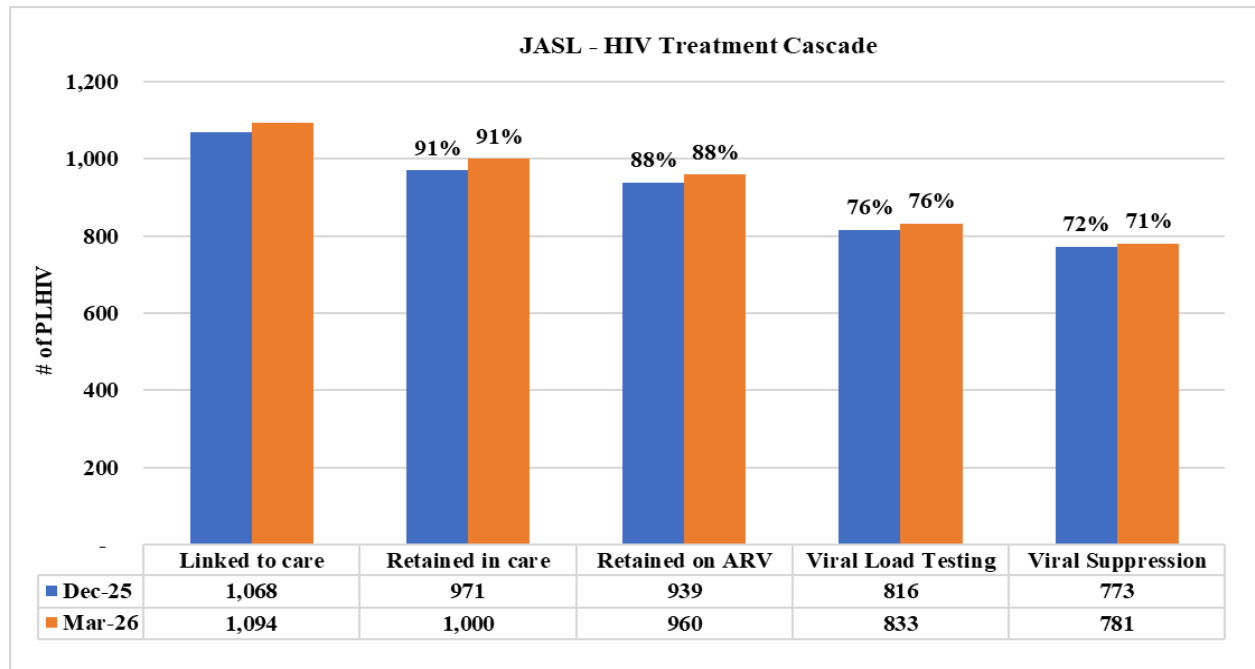


Figure 18: JASL- HIV Treatment Cascade

March 2026, when compared to December 2025, retained in care, retention on ARV and viral load testing remained the same. However, viral suppression declined by 1%. JASL continues to experience challenges with viral load result processing turnaround times. They have been engaging with NPHL directly to retrieve results in an effort to mitigate further impact on the treatment cascade (Figure 18).

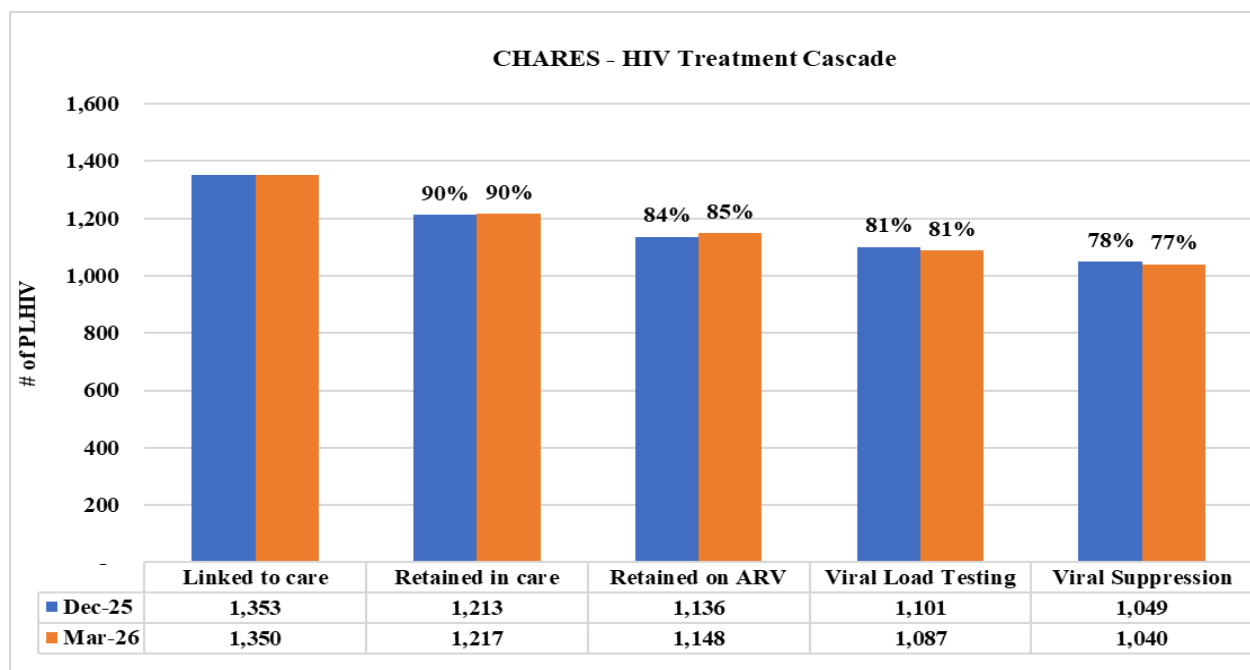


Figure 19: CHARES - HIV Treatment Cascade

March 2026, when compared to December 2025, retained in care remained the same, retained on ARV increased by 1%, viral load testing remained the same, and viral suppression declined by 1% (Figure 19).

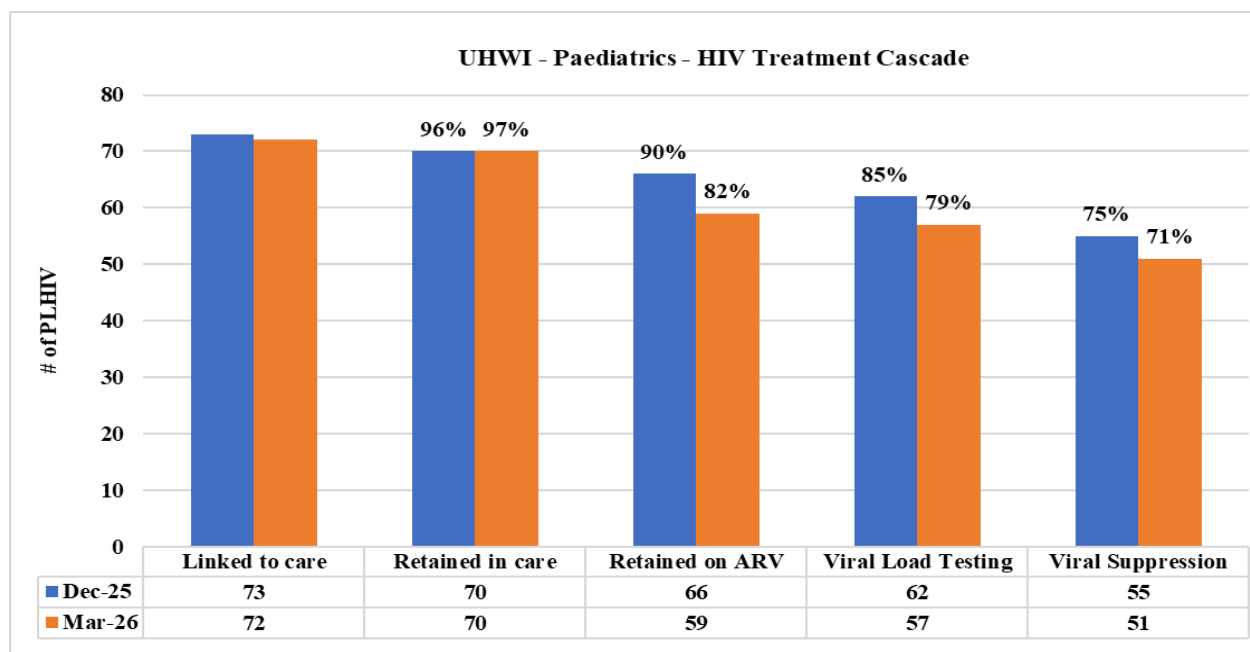


Figure 20: UHWI - Paediatrics - HIV Treatment Cascade

March 2026, when compared to December 2025, retained in care increased by 1%, retained on ARV declined by 8%, viral load testing declined by 6%, and viral suppression increased by 4% (Figure 20).

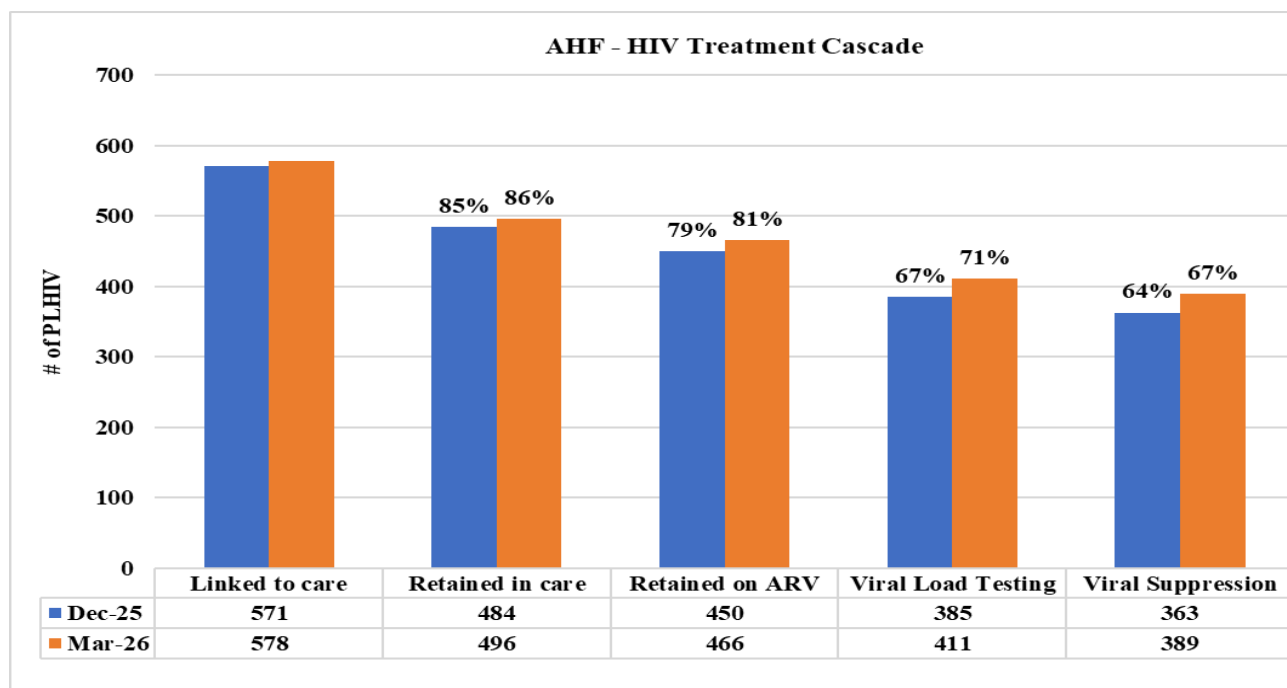


Figure 21: AHF - HIV Treatment Cascade

March 2026, compared to December 2025, retained in care increased by 1%, retained on ARV increased by 2%, viral load testing increased by 4%, and viral suppression increased by 3% (Figure 21).

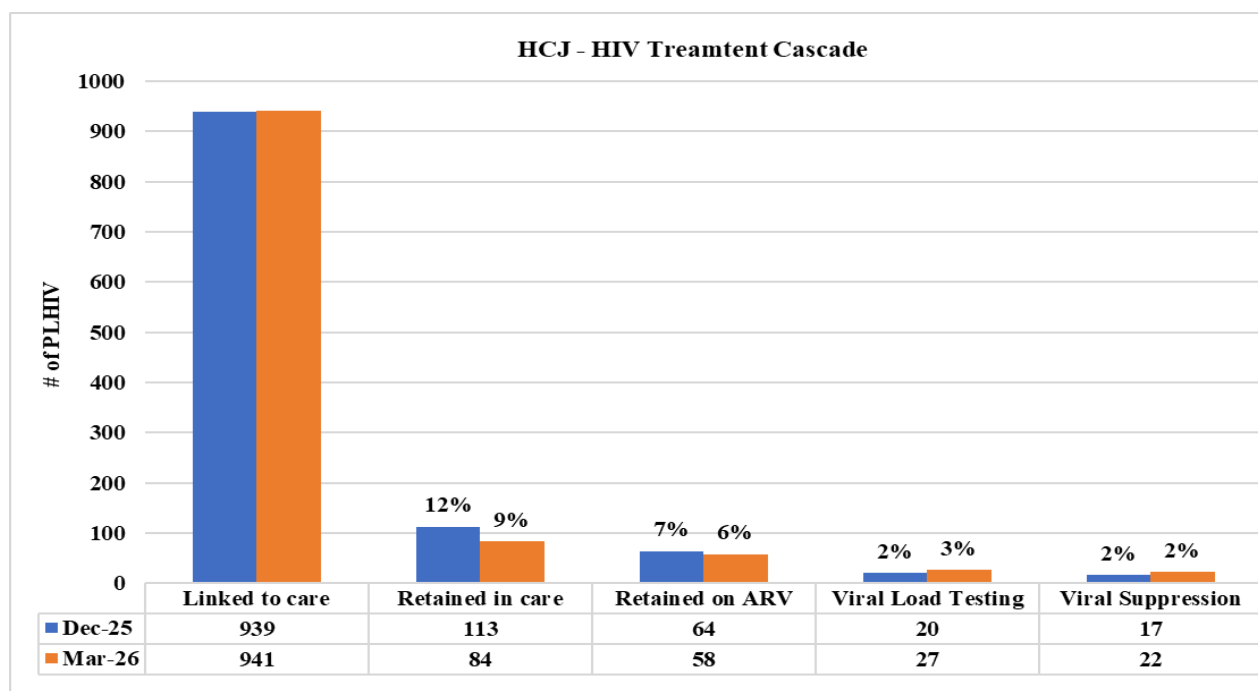


Figure 22: HCJ - HIV Treatment Cascade

March 2025, when compared to December 2025, retained in care declined by 61%, retained on ARV declined by 42%, viral load testing declined by 31%, and viral suppression declined by 29% (Figure 22).

Table 17: HCJ Doctors who are entering in TSIS as of March 2026

HCJ Doctors	# of PLHIV retained in care
Dr Audley McConnell - PRIVATE Practice	3
Dr. Andrea Purai -Medical Centre	2
Dr. Camille Campbell -Medical Centre	6
Dr. Clive Anderson -Medical Centre	1
Dr. Clive McKenzie -Medical Centre	1
Dr. Dane Levy -Medical Center	1
Dr. Dilini Chen -Medical Centre	2
Dr. Garfield Chapman -Medical Centre	3
Dr. Jeneen Jackson-Matthews -Medical Centre	3
Dr. Jodi-Ann Jolly -Medical Centre	1
Dr. Jodine Tafari -Medical Centre	2
Dr. Kadiann Pinnock -Medical Centre	1
Dr. Lisa McPherson -Medical Centre	1
Dr. Matthew Bird -Medical Centre	1
Dr. Maxine Cargill -Medical Centre	3
Dr. Micas Campbell -Medical Centre	1
Dr. Otaniyenuwa Asemota -Medical Centre	11
Dr. Peter Figueroa -Medical Centre	2

HCJ Doctors	# of PLHIV retained in care
Dr. Peter Swaby -Medical Centre	4
Dr. Ravi Aavvaru -Medical Centre	1
Dr. Richard Watson -Medical Centre	7
Dr. Rogelia Wallace -Medical Centre	2
Dr. Rosemary King -Medical Centre	4
Dr. San Lwin -Medical Center	5
Dr. Sophia Carlis-Campbell -Medical Centre	3
Dr. Suzanne Arscott -Medical Centre	1
Dr. Swane Gardener -Medical Centre	1
Dr. Tana Ricketts Roomes -Medical Centre	2
Dr. Tina Hylton-Kong -Medical Centre	5
Dr. Vanessa Hart -Medical Centre	2
Dr. Yochel Samuels-Williams -Medical Centre	2
Grand Total	84

Table 17 highlights 31/68 (46%) of HCJ doctors who are still entering in TSIS as of March 2026.

Table 18: PLHIV Retained on ARV as of March 2026 Matched Against the Annual Target

Region	Parish	On ARV 2025 Target	PLHIV Retained on ARV	Percentage achieved
SERHA	St. Thomas	366	307	84%
	Kingston and St. Andrew	3,780	3,138	83%
	St. Catherine	2,236	1,870	84%
WRHA	Trelawny	407	368	90%
	Hanover	246	220	89%
	Westmoreland	767	708	92%
	St. James	2,090	1,870	89%
NERHA	Portland	432	315	73%
	St. Mary	653	482	74%
	St. Ann	1,188	854	72%
SRHA	Clarendon	979	851	87%
	Manchester	978	851	87%
	St. Elizabeth	383	346	90%
	Total	14,505	12,180	84%

The total number of patients on ARV across the parishes as of March 2026 was 12,180. As of March 2026, no parish has met the On ARV target. However, it is important to note that Trelawny, Westmoreland and St. Elizabeth have achieved a percentage of 90 or more thus far (Table 18).

Table 19: PLHIV Viral Load Testing of those eligible for a viral load test as of March 2026, matched against the Annual Target

Target: 95%

Region	Parish	Viral Load Testing Achieved	Viral Load Testing Target	Percentage Achieved
SERHA	St. Thomas	286	293	98%
	Kingston and St. Andrew	2,381	3,059	78%
	St. Catherine	1,629	1,827	89%
WRHA	Trelawny	324	358	91%
	Hanover	197	216	91%
	Westmoreland	451	693	65%
	St. James	1,521	1,833	83%
NERHA	Portland	284	308	92%
	St. Mary	454	475	96%
	St. Ann	728	833	87%
SRHA	Clarendon	687	830	83%
	Manchester	668	829	81%
	St. Elizabeth	217	334	65%
	Total	9,827	11,888	83%

As of March 2026, St. Thomas and St. Mary have surpassed the viral load testing target of 95%. The viral load testing being below the target at 83% was attributed to samples being compromised, viral load results not being entered correctly in TSIS, and patients not wanting to do their viral load test. The data quality issue was further investigated, and a meeting was scheduled with the TCSOs to enter missing viral load data and investigate those without a viral load test done. The outcome of this plan of action will be reviewed in the next TSIS Data Quality scheduled for April 2026 (Table 19).

Table 20: PLHIV Viral Suppression of those eligible for a viral load test as of March 2026, matched against the Annual Target

Target: 95%

Region	Parish	Viral Suppression Achieved	Viral Suppression Target	Percentage achieved
SERHA	St. Thomas	270	293	92%
	Kingston and St. Andrew	2,174	3,059	71%
	St. Catherine	1,418	1,827	78%
WRHA	Trelawny	296	358	83%
	Hanover	187	216	87%
	Westmoreland	404	693	58%
	St. James	1,394	1,833	76%
NERHA	Portland	265	308	86%
	St. Mary	414	475	87%
	St. Ann	663	833	80%
SRHA	Clarendon	616	830	74%
	Manchester	620	829	75%
	St. Elizabeth	200	334	60%
	Total	8,921	11,888	75%

As of March 2026, no parish has met or surpassed the viral suppression target of 95% (Table 20).

Monitoring and Evaluation

The table below indicates that special attention is needed for Albert Town, Black River Antenatal, Falmouth Hospital, Junction Health Center, Mandeville Hospital Antenatal, May Pen Antenatal, Metcalfe Street Juvenile Remand Centre, Santa Cruz Health Center, Spalding Health Center, Spanish Town Hospital-Antenatal, St. Ann's Bay Hospital-Antenatal and Victoria Jubilee Hospital.

Table 21: Treatment cascade gap analysis

Key:

Met/surpassed national target ($\geq 95\%$)

Moderate performance (70% - 94%)

Underperformance (50% - 69%)

Critical Underperformance ($\leq 50\%$)

Treatment Site	In Care/ Linked	On ARV/In Care	VL Testing/On ARV	Virally suppressed/VL Testing
AIDS Healthcare Foundation - Hagley Park	86%	94%	88%	95%
Albert Town Health Center	100%	100%	33%	100%
Alexandria Health Center	94%	90%	93%	74%
Bellevue Hospital	95%	72%	100%	100%
Black River Antenatal	0%	0%	0%	0%
Black River Health Center	78%	92%	66%	93%
Black River Paediatrics	100%	100%	100%	67%
Buff Bay Health Center	91%	100%	90%	91%
Bustamante Hospital - Paediatric	100%	96%	88%	90%
Catherine Hall Health Center	94%	100%	87%	100%
Chapelton Health Center	87%	93%	72%	87%
CHARES	90%	94%	95%	96%
Claremont Center of Excellence	88%	93%	93%	85%
Comprehensive Clinic - Adults	74%	95%	72%	93%
Comprehensive Clinic - Paediatric	94%	92%	95%	64%
Cornwall Regional Hospital	89%	96%	78%	87%
Cornwall Regional Hospital - Paediatric	92%	100%	75%	78%
Darliston Health Center	100%	100%	75%	100%
Duhaney Park Health Center	87%	95%	85%	95%
Duncans Health Center	96%	99%	89%	91%
Falmouth Hospital	50%	0%	0%	0%
Greater Portmore Health Center	91%	95%	89%	91%
Horizon Adult Remand Center	100%	100%	88%	100%

Treatment Site	In Care/ Linked	On ARV/In Care	VL Testing/On ARV	Virally suppressed/VL Testing
JASL - Kingston	93%	96%	90%	95%
JASL - Montego Bay	87%	97%	76%	91%
JASL - St. Anns Bay	91%	97%	89%	93%
Junction Health Center	100%	100%	0%	0%
Kingston Public Hospital	83%	97%	88%	90%
Linstead Health Center	95%	97%	88%	87%
Lionel Town Health Center	86%	92%	82%	94%
Lucea Health Center	95%	99%	90%	95%
Manchioneal Health Center	100%	100%	100%	100%
Mandeville - Paediatrics	100%	83%	60%	100%
Mandeville Hospital	89%	99%	82%	94%
Mandeville Hospital - Antenatal	0%	0%	0%	0%
Mandeville Type 5 Health Center	90%	96%	79%	93%
Maxfield Park Health Center	83%	92%	82%	90%
May Pen Antenatal	33%	100%	50%	100%
May Pen H/C - Adult	81%	94%	82%	90%
May Pen Hospital - Paediatric	60%	100%	67%	100%
Metcalfe Street Juvenile Remand Center	100%	100%	100%	0%
Montego Bay Type 5 Health Center	89%	98%	86%	95%
Morant Bay Health Center - Adult	92%	99%	93%	95%
National Chest Hospital	91%	89%	69%	92%
Negril Health Center	100%	100%	75%	67%
Old Harbour Health Center	89%	96%	83%	81%
Port Antonio Health Center	92%	97%	91%	94%
Port Maria Health Center	93%	96%	94%	91%
Princess Margaret Hospital	100%	100%	100%	67%
Richmond Farm - Adult	100%	100%	100%	100%
Santa Cruz Health Center	88%	90%	5%	0%
Sav la Mar Hospital - Paediatrics	100%	100%	67%	100%
Sav-la-mar Health Center	93%	94%	72%	78%
Sav-la-mar Hospital	76%	91%	62%	93%
South Camp Road - Adult	100%	100%	100%	100%
Spalding Health Center	100%	100%	33%	0%
Spanish Town Hospital - Antenatal	22%	0%	0%	0%
Spanish Town Hospital - Paediatric	94%	100%	73%	82%
St Ann Bay Hospital - Antenatal	50%	100%	50%	100%
St. Ann's Bay Health Center	78%	96%	85%	93%
St. Ann's Bay Hospital - Paediatric	85%	100%	82%	71%
St. Catherine Correctional Center - Adult	100%	96%	85%	83%
St. Jago Park - Adolescent	90%	91%	89%	86%

Treatment Site	In Care/ Linked	On ARV/In Care	VL Testing/On ARV	Virally suppressed/VL Testing
St. Jago Park Health Center	81%	97%	88%	87%
Tamarind Farm - Adult	100%	100%	100%	100%
Tower Street - Adult	100%	100%	89%	79%
University Hospital - Paediatric	97%	84%	97%	89%
Victoria Jubilee Hospital	49%	81%	48%	93%
Windward Road Health Center	88%	95%	70%	90%

STI

STI Cases Reporting Compliance

In the quarter spanning January to March 2026, a total of 327 reports were expected across the four regions; however, only 162 were received, resulting in an overall reporting rate of 50%. All submitted reports were received on time, yielding an on-time submission rate of 50%. The North East Region demonstrated the highest performance by submitting 32 of 39 expected reports (82%), with all submissions made on time (Table 22).

The Western Region followed, submitting 44 of 54 reports (81%), all of which were on time. The Southern Region submitted 58 of its 75 expected reports (77%), all within the required timeframe. The South East Region recorded the lowest performance, submitting only 28 of 159 expected reports (18%), which was received on time (Table 22).

Reporting rates are anticipated to improve throughout the year as recovery efforts continue following Hurricane Melissa, which made landfall in late October. However, targeted intervention is needed in the South East Region to enhance the completeness and timeliness of submitted reports, as late, or a lack of data entry hinders obtaining a true representation of STI cases recorded for the quarter (Table 22).

Table 22: Reporting Compliance for STI Cases - January to March 2026

Region	Expected Reports	Actual Reports	Percent Received	Reports On Time	Percent On Time
Western Region	54	44	81%	44	81%
North East Region	39	32	82%	32	82%
Southern Region	75	58	77%	58	77%
South East Region	159	28	18%	28	18%
Total	327	162	50%	162	50%

STI Surveillance

A total of 1,357 sexually transmitted infection (STI) cases were reported for the month of March, with 237 (17%) cases among males and 1,120 (83%) among females (Figure 22). Overall, there were 5,563 sexually transmitted infection (STI) cases reported during the January to March quarter, with the 4,523 occurring among females (81%) compared to 1,040 males (19%). Throughout the quarter, the proportion of male cases slightly increased by 2% from 18% in January to 20% in February before declining by 3% to 17% in March, indicating a downward trend, possibly due to late report submission. Conversely, female cases declined by 2% over the first two months (from 82% to 80%), reflecting reduced uptake in February, before increasing by 3% in March to 83%. Male cases in March were the lowest recorded for the quarter, while female cases reached their highest reporting rate.

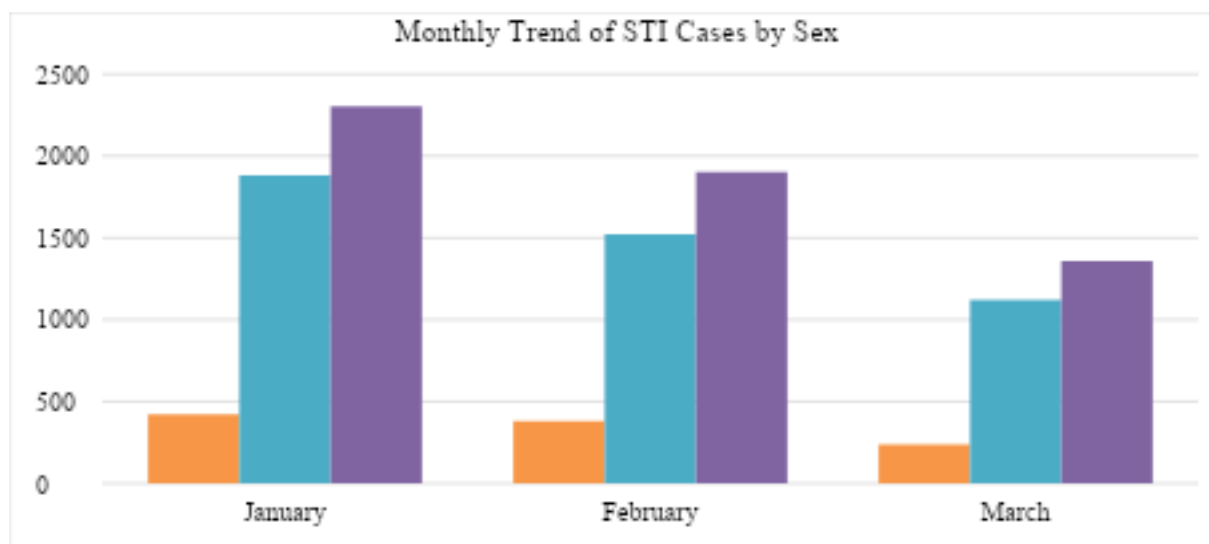


Figure 23: STI Cases by Sex – January to March 2026

Age-Group Disaggregation

A total of 1,357 sexually transmitted infection (STI) cases were reported for March, with 1,120 cases (83%) among females and 237 cases (17%) among males. The highest burden of STI cases was observed among individuals aged 20 to 39 years, accounting for 731 (54%) of total cases, with the 20-29 age group contributing the most with 415 cases (83 males, 332 females). STI cases were also significantly observed in older populations, with the 50-59 age group contributing 13% (174 cases), consisting of 28 males and 146 females. The 60-69 and 70+ age group contributed 4% (7 males, 53 females) and 1% (6 males, 16 females), respectively.

Among adolescents aged 15-19 years, 139 (14%) cases were reported (33 males and 106 females). Children under 15 years of age accounted for 34 cases, with a notable 23 cases (2%) among females aged 10-14.

There were also 11 cases (1%) in the 0-9 years' age group (7 females, 4 males). Ten unknown cases were reported, representing 1% of total STI cases for the reporting period (Figure 24).

From January to March, STI transmission remains predominantly concentrated among individuals aged 20–39, who consistently account for the largest share of cases. However, there is a notable shift in March with increased representation of cases among the 50+ population, likely due to improved age disaggregation. The DHIS2 database was updated in March to further disaggregate the 50+ age group into 50–59, 60–69, and 70+, enabling more precise analysis and representation of the country's ageing population. Additionally, the overall decline in cases in March across most age groups may be due to late reporting rather than a true decrease in transmission.

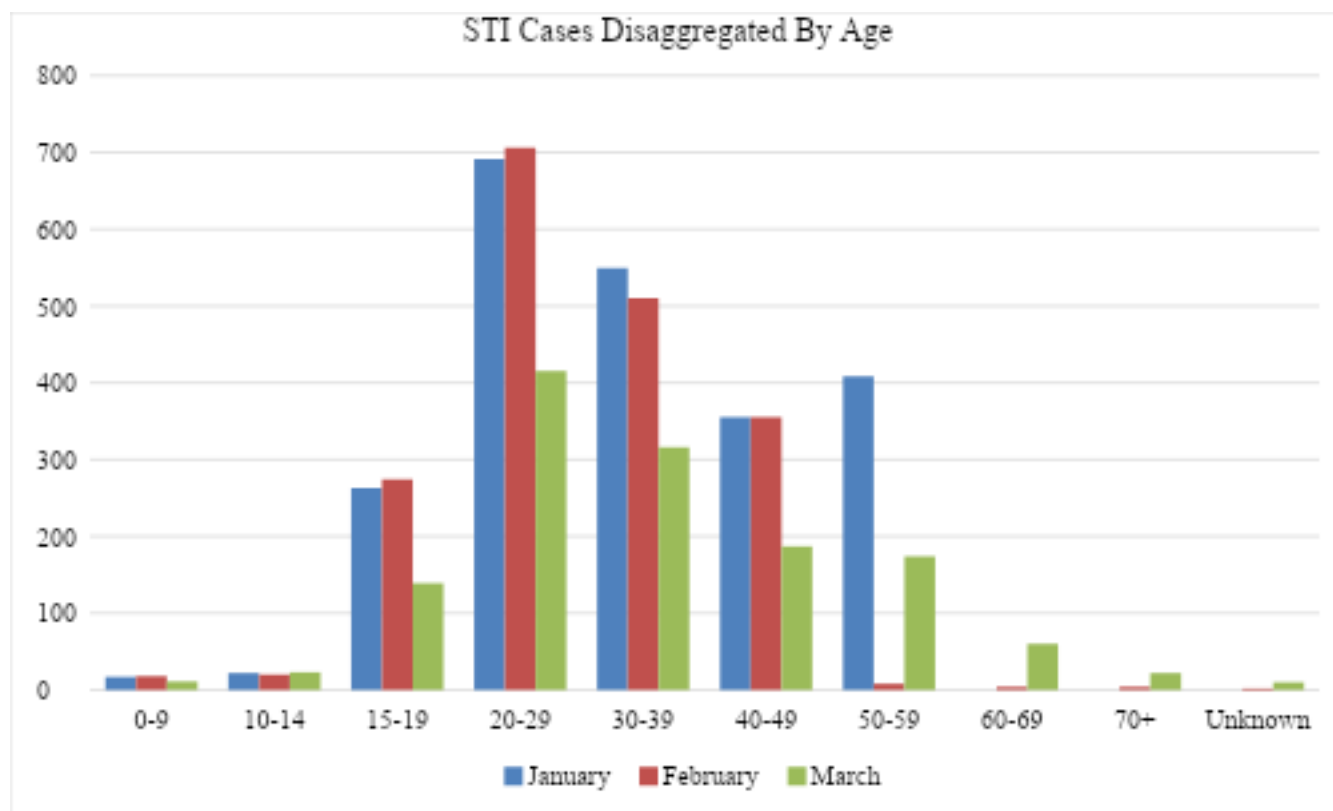


Figure 24: STI Cases Disaggregated by Age – January to March 2026

STI Breakdown

A. Genital Discharge Syndrome

A total of 1,147 cases of Genital Discharge Syndrome were recorded in March, with a significantly higher burden among females with 1,004 (88%) cases compared to 143 (12%) cases in males. Among these cases, genital discharge was the predominant presentation, affecting 1,103 individuals, of which 143 are males, and 960 are females, representing 96% of total cases reported. Cervicitis, a condition specific to females, accounted for 44 (4%) cases.

From January to March, Genital Discharge Syndrome cases steadily declined from 1,944 to 1,103. Cervicitis cases peaked in February at 106 before dropping sharply to 44 in March, contributing to an overall downward trend during the period (Figure 25).

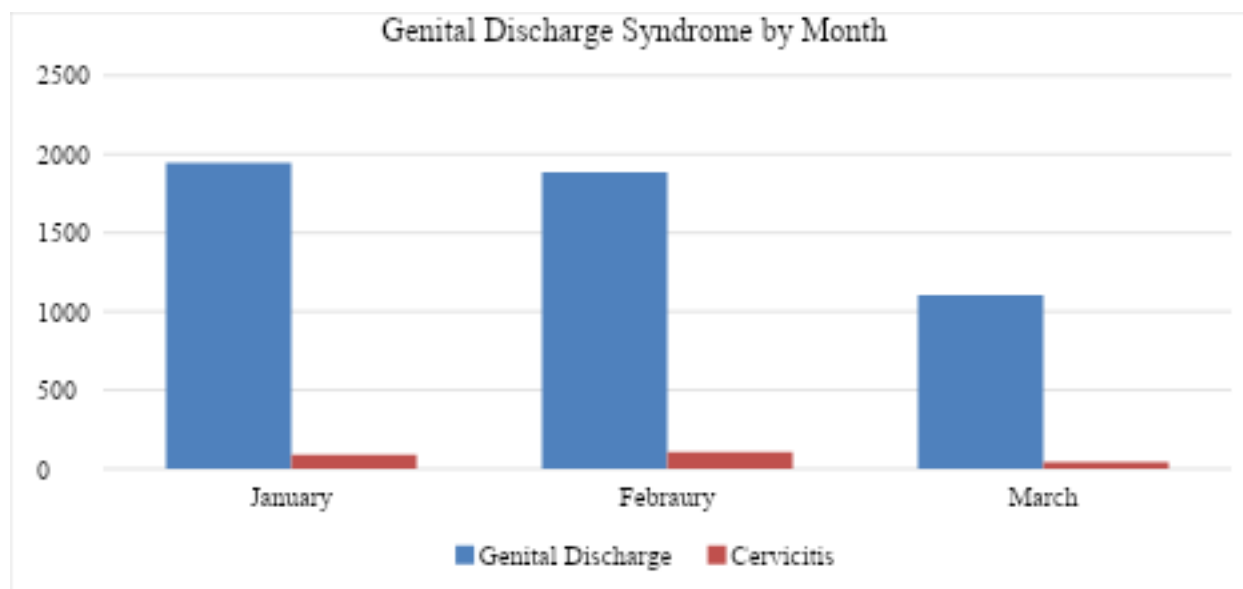


Figure 25: Distribution of Genital Discharge and Cervicitis Cases – January to March 2026

B. Genital Ulcer Disease (GUD)

A total of 27 Genital Ulcer Disease (GUD) cases were recorded during the reporting period. Genital herpes was the main aetiology identified, accounting for 92% of cases (25). Females represented 59% (16) of reported cases, while males accounted for 41% (11) cases. There was one female Chancroid case and one male Granuloma Inguinale case recorded for March, each representing 4% of total GUD cases (Figure 26).

During the January to March quarter, Genital Herpes remained the most frequently reported condition. However, a decline in cases in March contributed to an overall reduction in Genital Ulcer Disease Syndrome during the period.

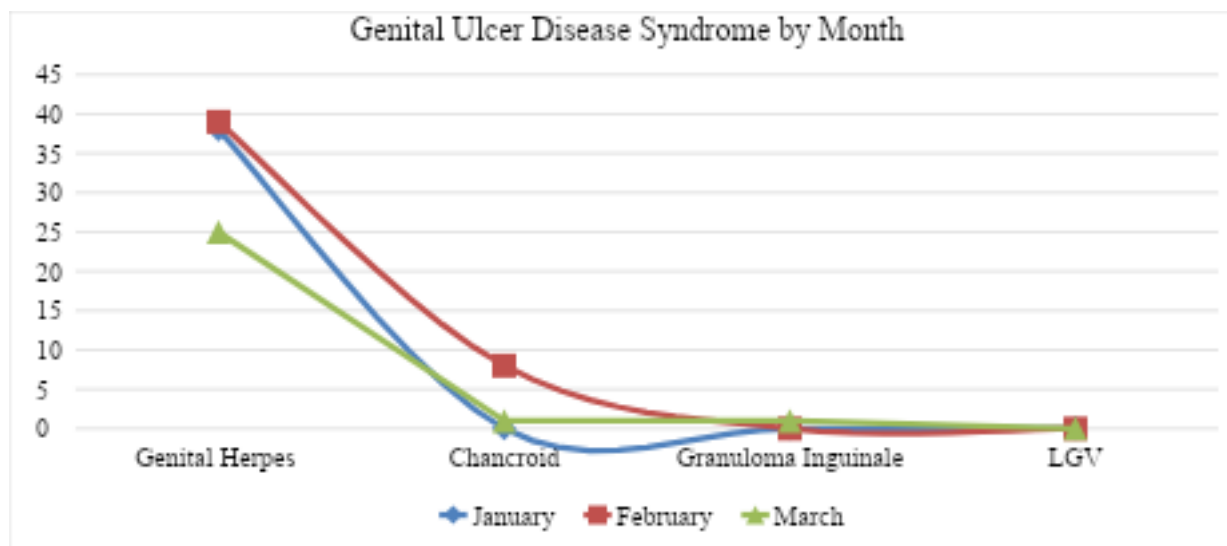


Figure 26: Distribution of Genital Ulcer Disease Syndrome Cases – January to March 2026

C. Syphilis Cases

A total of 97 syphilis cases were reported in March, with a slightly higher burden observed among females (49 cases) compared to males (48 cases). Over the quarter, syphilis cases were increasingly driven by latent-stage infections. Late latent remained the largest category, rising from 56% (40) in January to 57% (47) in February and 67% (65) in March. Early latent also increased slightly from 18% (13) to 21% (17) and 19% (18).

In contrast, secondary syphilis declined notably from 21% (15) in January to 13% (11) in February and 6% (6) in March. Primary cases remained relatively stable at 6% to 7%. Tertiary cases were minimal but increased slightly from 0% in January to 2% (2) in March, while congenital cases remained at 0% throughout (Figure 26). The shift towards late latent diagnoses may be due to delayed detection.

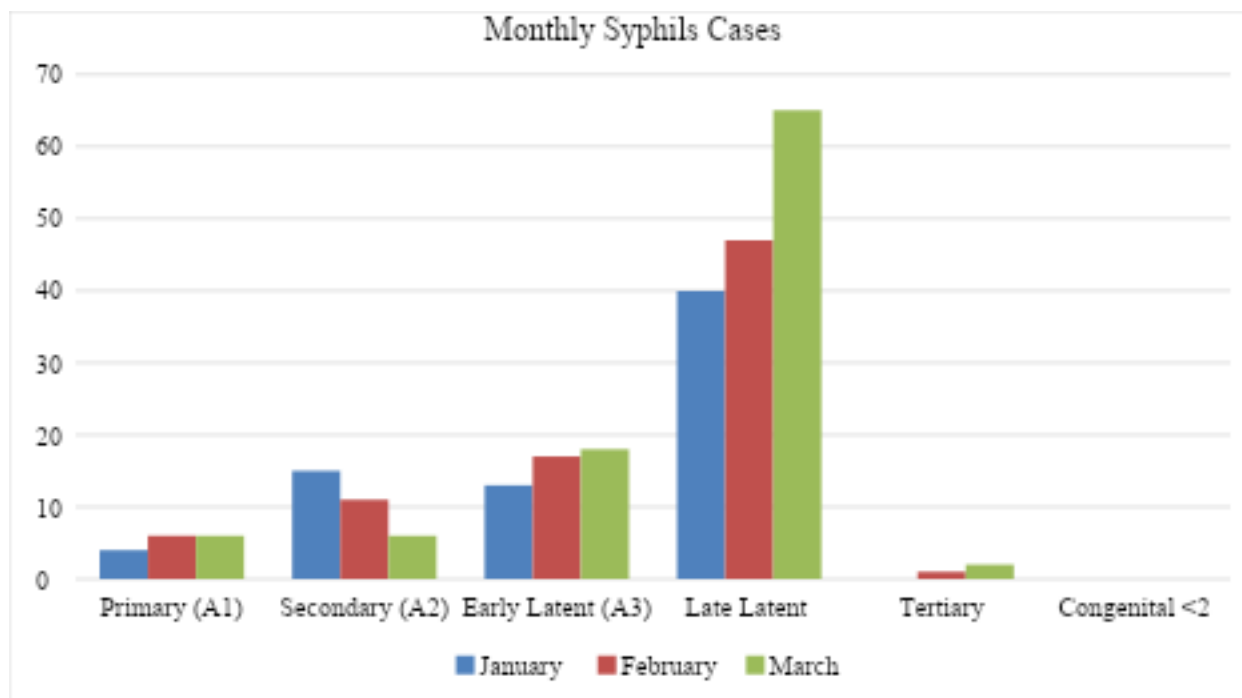


Figure 27: Distribution of Syphilis Cases – January to March 2026

D. Other STI and Non-STI Conditions

A total of 103 individuals received diagnosis and treatment for various sexually transmitted infections (STIs) and related conditions, with 61% among females (63 cases) compared to 39% for males (40 cases).

Epidemiological treatment was given to 41 individuals. Of these, 11% (11) received treatment for syphilis exposure, while 29% (30) were treated for gonorrhoea or chlamydia exposure.

Pelvic Inflammatory Disease (PID) was the most reported condition, comprising 31% of total cases and occurring exclusively among females (32 cases). Among males, epididymo-orchitis was recorded in 3 cases (3%).

Genital warts accounted for 17% of reported cases, affecting 17 persons (11 females and 6 males). Ophthalmia Neonatorum represented 7% of total cases, with 3 females and 4 males affected. Two cases (1 male, 1 female) of Hepatitis B/C were recorded, representing 2% of total cases reported. There was one female case (1%) of Human T-lymphotropic Virus I/II reported during the period (Figure 28).

During the January to March quarter, there was variability in reported STI and non-STI conditions. PID declined sharply from January to February before increasing in March. Genital warts peaked in February and then decreased in March. Meanwhile, partner treatment for GC/Chlamydia and syphilis increased steadily into March, pointing to possible improved case-finding toward the end of the quarter.

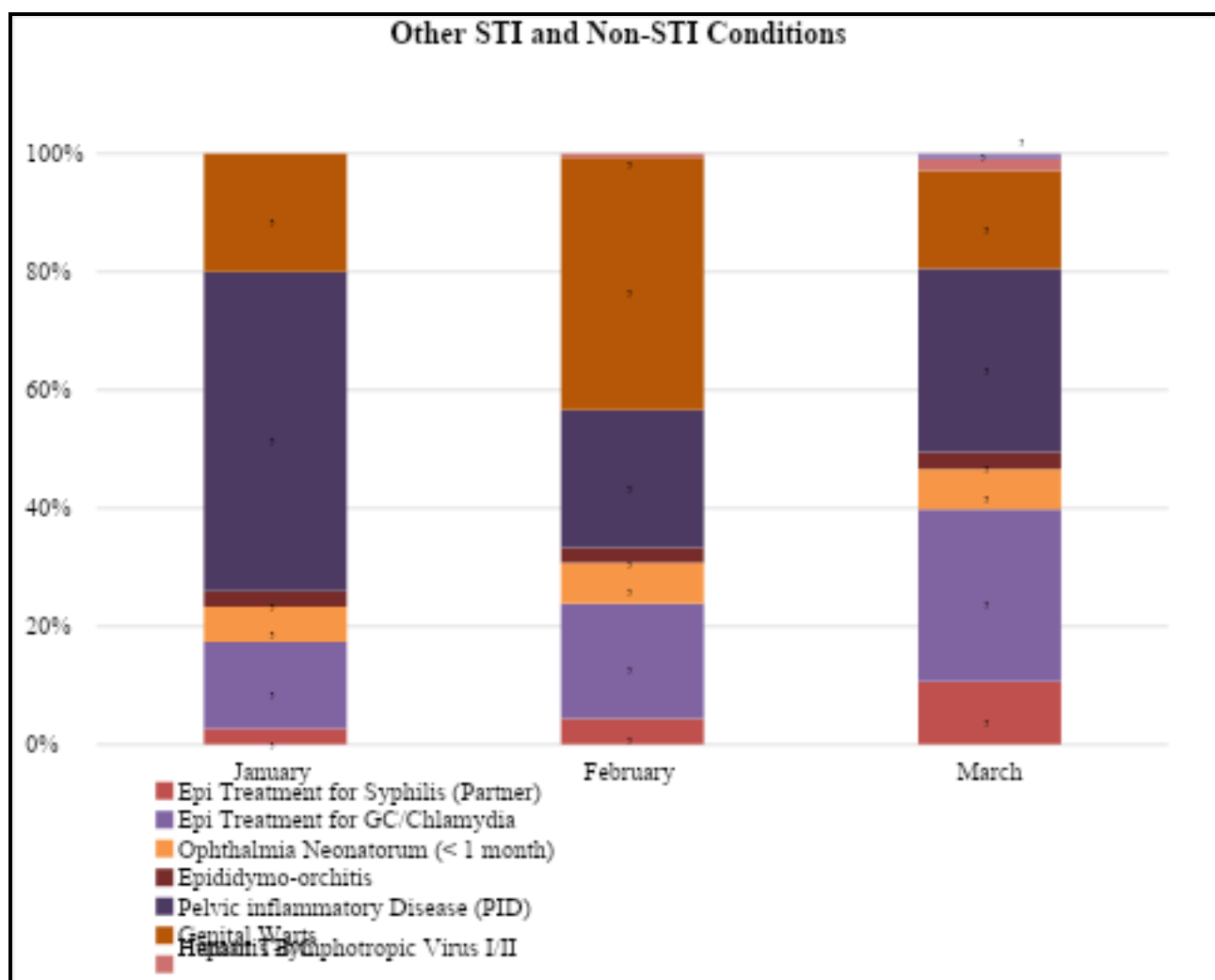


Figure 28: Distribution of Other STI and Non-STI Conditions – January to March 2026

STI Clinic Testing and Surveillance

During the reporting period, a total of 5,694 individuals attended STI clinics, comprising 1,286 males (23%) and 4,408 (77%) females (Table 23).

HIV Testing Coverage and Yield

HIV testing was conducted for 2,941 attendees, reflecting an overall testing coverage of 52%. Males demonstrated a notably higher HIV testing rate, with 909 out of 1,286 males tested (71%) compared to 2,032 out of 4,408 females (46%).

Despite the higher testing coverage among males, the HIV positivity yield remained low at 2% (14 positive cases). However, females produced an even lower positivity rate of 1% despite having similar number of positive cases (14 positive cases). As a result, the overall positivity rate was 1%; 28 cases (Table 23).

Syphilis Testing Coverage and Yield

Syphilis testing coverage reached 49%, with 2,809 individuals tested among the total clinic attendees during the reporting period. Testing uptake was higher among males, with 871 of 1,286 (68%) tested, compared to 1,938 of 4,408 females (44%).

There was a reported syphilis positivity yield of 2%, representing a total of 66 cases, 30 among males (3%) and 36 cases among females (2%).

Greater testing efforts may be critical, as both males and females accounted for a similar proportion of HIV cases during the January to March reporting period. While females accounted for six more syphilis cases than males, the yield among males remains slightly higher (Table 23).

Table 23: STI Clinic Testing and Surveillance - January to March 2026

Sex	STI Clinic Attendees	Tested for HIV (%)	Positivity Yield (HIV)	YTD Positivity Yield (HIV)	Tested for Syphilis (%)	Positivity Yield (Syphilis)	YTD Positivity Yield (Syphilis)
Males	1,286 (23%)	909 (71%)	14 (2%)	14/909 (2%)	871 (68%)	30 (3%)	30/871 (3%)
Females	4,408 (77%)	2,032 (46%)	14 (1%)	14/2,032 (1%)	1,938 (44%)	36 (2%)	36/1,938 (2%)
Total	5,694	2,941 (52%)	28 (1%)	28/2,941 (1%)	2,809 (49%)	66 (2%)	66/2,809 (2%)

Tuberculosis

TB Surveillance

Surveillance Data (YTD) - Latest MOHW Epi Weekly Report (Monthly)

As of March 2026, the surveillance data shows:

Table 24: Number of TB Cases Confirmed

Event	YTD 2026	YTD 2025

Tuberculosis	4	14
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As of March 2026, the TB Database Register contains approximately 112 clients, the majority of whom were confirmed in 2025.

HEALTH INFORMATION SYSTEMS

- 1. TSIS Help Desk Functions**
 - a. Handling site users' queries (ongoing activity)
 - User Access TSIS Data Entry/TSIS Reports
 - User creation and modification, TSIS Data Entry/TSIS Reports (17 new accounts were created)
 - b. Support data cleaning activities (ongoing activity)

Table 25: Summary of Data Cleaning Activities

	January-March 2026	YTD Total
Transfers	166	166
Mergers	30	30
Deletions	63	63
Case Closure (Protocol)	111	111
Case Closure (Deaths)	76	76

d. Ad-hoc reports were created to facilitate data queries for internal and external stakeholders

Including:

1. EPI death reconciliation dataset completed
2. SI Data Quality Audit Backend Pull
3. HCJ Dataset
4. Data Collection Tool Dataset completed

2. Databases

Refers to all the applications currently under the purview of the SI Technical Team:

1. EMTCT - DHIS2 Platform maintained
2. EMTCT - Online hospital registers (HEI, Maternal HIV, Maternal Syphilis) maintained.
3. STI and CI Monthly reports - DHIS2 Platform:
 - a. On-going help desk support to staff
 - b. Participated in review of STI audit tool
4. Tuberculosis Database:
 - TB Database is live, SI continues ongoing calls to facilitate on boarding and subsequent data entry
 - The TB User Guide is still a work in progress
5. Prevention Database:
 - Provided ongoing end-user support.
 - Updated a program rule that prevented saving and/or editing a client's record whenever it ran.
 - Compiled data for
 - o Global Fund Payment for Result (PFR)
 - o SI Report
 - o Grants Progress Report
 - o MOF Report
 - De-duplication report sent to CSOs and RHAs
6. Revised EMTCT/STI/CI Monthly Summary forms to be implemented in January 2026
 - Create temporary users and organization unit for testing of the revised reports
 - Introduction of revised EMTCT monthly report for 2026 to stakeholders' island wide

- Orientation session held with three nurses to the EMTCT DHIS2 Platform (VJH - 2, Spanish Town Hospital - 1)
- All stakeholders trained and feedback given
- Currently modifying the forms based on feedback
- Updated all new indicators for 2026 reporting
- Updated New EMTCT with validation rules

3. Other Activities

- a. SMS broadcasts to support retention activities target clients who have missed scheduled appointments.
- b. Broadcast general health messaging to clients linked to treatment sites impacted by Hurricane Melissa, providing guidance on accessing care and maintaining treatment continuity.
- c. Support prevention component by conducting data quality inspection on their recorded dataset
- d. HealthJam remains active, supported by a series of training initiatives aimed at equipping MOHW staff with the skills to manage and maintain the platform.
- e. The EEHR platform is online, with all necessary updates completed.
- f. Continue to cross reference NHF ARV distribution lists with TSIS clients' names:
 1. to update TSIS with ARV medication information
 2. to determine persons with an appointment who received ARV medication

4. Maintenance Report

- TSIS and EMTCT data continues to be backed up weekly, while all other databases are backed up monthly.
- The full migration of TSIS has been transitioned to a consultancy to facilitate platform upgrades and the integration of prevention and treatment databases, supporting the development of an interoperable framework. The selection process for the consultancy is in progress, with the project expected to span six months
- The HealthJam application supported by C-Tech is housed on MOHW server; the application is online and currently being used in two (2) regional authorities, at the following sites: Duncans Health Centre and Port Maria Health Centre. There is a steady uptake of the platform with both sites recording the successful registration of over 20+ PLHIV since pilot. We are currently in ongoing meetings to have a Whatsapp integration to interact with clients which can be an easier approach than regular SMS messaging. This was a request made between the interactions with some clients through TextIT SMS feature.
- TSIS2 Reports: Software tools required to perform new deployment include Windows Server 2022(supplied by ICTU), Microsoft SQL Server (already procured) and Full-Convert (recently procured).
- DHIS Prevention upgraded version in use.

- DHIS Treatment upgraded version in use.

5. Security Check Report

The SI technical team is responsible for maintaining user access listings for TSIS and TSIS Reports. In accordance with the internal SOP, all access requests are processed within five business days. 756 accounts exist on TSIS of which 514 are active; for the period January to March 2026.

6. SMS Usage Report

Table 26: SMS Uptake in TSIS as at 10-04-2026

TSIS clients with notification number	Included to receive SMS	Excluded to receive SMS
2,754	2,497	257

This table shows the uptake of SMS reminder notification for PLHIV registered on the TSIS platform.

SI ACTIONS FROM TECHNICAL WORKING GROUP MEETINGS

Table 27: SI Actions Technical Working Group Meetings

Meetings	Actions	Responsible Officer	Status
TSIS Data Quality Meeting	Data Quality Action Packs for Data Quality Improvement.	TCSOs and SI	Mergers, transfers, case closures, and related updates have been sent into SI and are being carried out where applicable. Data Quality Action Packs were sent out to the regions and CSOs for action and progress to be presented in the April 2026 meeting.
PMTCT Data Review Meeting	Challenges with data entry at Victoria Jubilee Hospital to be investigated and actioned. Follow up on the 2 laptops have been requested for 2 PMTCT nurses for the WRHA to facilitate PMTCT data entry. Database updates will be implemented to introduce a data completeness cut-off date, ensuring that reports submitted on time are not affected when subsequently re-accessed for revisions or updates.	Strategic Information Component HIV/STI/TB Unit Strategic Information Component	Completed Completed Ongoing
WRHA Supportive Supervision	Cross-referencing demographic data, aligning first clinic and clinician exam dates, and ensuring accurate entry of first ARV date, first CD4 date, and most recent viral load date. Conduct monthly data monitoring activities to identify gaps and assist sites in developing remedial actions.	Strategic Information Component, Focal Point Strategic Information Component, Focal Point	Ongoing Ongoing

Meetings	Actions	Responsible Officer	Status
NERHA Supportive Supervision	Continue return-to-care campaigns and cross-check patients suspected of private care against NPHL viral load line lists to update records accurately.	TCSO and SI	Ongoing
	Compile and submit line lists of long-outstanding viral load results to TCS for follow-up with NPHL to improve turnaround time.	TCSO and TCS	Ongoing
SRHA Supportive Supervision	Monthly gap analysis to identify gaps in data for Paediatric sites with frequent lost to follow-ups.	Strategic Information Component, Focal Point	Ongoing
SERHA Supportive Supervision	Sensitization session with the SI audit team to be held on March 23, 2026.	TCSO and SI	Completed
	Data reconciliation meeting to be held on March 23, 2026.	TCSO and SI	Completed
Bellevue Hospital	Receive, review, and act on updated list of patient data discrepancies from new TSIS pull.	Strategic Information Component	Completed
	Schedule follow-up meetings to assess progress on all action items.	Dr. Andre Williams and SI - Data Entry Clerk	Completed
University Hospital Paediatrics (UHWI Paeds)	Conduct routine data cleaning and monitoring activities and provide feedback on all action items.	SI - Data Entry Clerk	Ongoing
Department of Correctional Services (DCS)	Implement quarterly data cleaning to verify patient status.	DCS & SI - Data Entry Clerk	Completed
	Establish a formal transfer protocol for patients moving between correctional facilities to ensure care continuity.	DCS & SI - Data Entry Clerk	Ongoing

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Strategic Operational Plan

Major Initiatives/ Activities	Key Outputs	Performance Measures/Indicators	Targets (Projections)	
			Jan-Mar	Status
M&E Technical Working Group Meeting	Action sheets for 4 M&E TWG meetings held with key stakeholders	4 M&E meeting reports submitted by Q4	N/A	Deferred to new fiscal year
Produce Global AIDS Monitoring (GAM) Report	Preparation and submission of the GAM & NCPI Report	Draft GAM & NCPI Report for 2025 submitted to MPD by Q4	Draft GAM & NCPI Report for 2025 submitted to MPD by March 31, 2026	The draft report has been submitted to MPD. The final approved report will be shared once all corrections and feedback from MPD have been addressed. Completed
SPECTRUM Report	Preparation & submission of the SPECTRUM Report	SPECTRUM Report submitted to UNAIDS by Q4	Data collation completed and submitted for SPECTRUM	

Major Initiatives/ Activities	Key Outputs	Performance Measures/Indicators	Targets (Projections)	
			April - June	Status
Strategic Information Training Report	Approved Annual Strategic Information training report	Annual Strategic Information training report (calendar year) submitted by Q4	N/A	
Global AIDS Monitoring (GAM) report	Global AIDS Monitoring (GAM) report	GAM report approved by the CMO for submission to Executive Management by Q4	N/A	
Quarterly HIV monitoring cascade report	Quarterly HIV monitoring cascade report	4 Quarterly HIV monitoring cascade reports	Approved Quarterly HIV monitoring cascade report (Jan-Mar) submitted	On track for completion for Q1
Quarterly STI Database Report	Quarterly STI Database Report	4 Quarterly STI Database reports	Approved Quarterly STI Database report submitted	On track for completion for Q1
Quarterly PMTCT Database Report	Quarterly PMTCT Database Report	4 Quarterly PMTCT database reports	Approved Quarterly PMTCT database report submitted	On track for completion for Q1
Quarterly HIV Prevention Database report	Quarterly HIV Prevention Database report	4 Quarterly HIV Prevention Database reports	Approved Quarterly HIV Prevention Database report submitted	On track for completion for Q1
Quarterly TB Database Report	Quarterly TB Database Report	4 Quarterly TB Database reports	Approved TB Database report submitted	On track for completion for Q1
Quarterly EEHR Dashboard Report	Quarterly EEHR Dashboard Report	4 Quarterly EEHR dashboard reports	N/A	

Major Initiatives/ Activities	Key Outputs	Performance Measures/Indicators	Targets (Projections)	
			April - June	Status
Data Repository Updates	Updating of the Data Repository	Number of updates on published and unpublished studies/reports (including STIs and TB)	N/A	
Revision of Database User Manuals	Database User Manuals revisions	Updated User Manuals for the Prevention and TSIS database	Developing and Printing of Database User Manuals for PMTCT, STI/CI and TB	In progress
Conduct research on HIV/STI/TB	Operational Research abstracts	2 Operational Research abstracts approved	N/A	
Assessment of HIV Data Quality	HIV Data Quality Audit reports	HIV DQA was conducted with at least 60% of corrective actions implemented	HIV Data Quality Audits conducted for NERHA, SRHA, SERHA Approved Audit Action Plan implementation report submitted (2025 audits approved)	On track for completion for Q1
PMTCT Data Review Meeting	PMTCT Data Review Meeting Reports with corrective action plans	4 PMTCT data review meeting reports	Approved PMTCT Data Review report submitted	On track for completion for Q1

Major Initiatives/ Activities	Key Outputs	Performance Measures/Indicators	Targets (Projections)	
			April - June	Status
Development of TB Data Quality Audit tool	TB Audit tool developed	TB data quality audit tool pre-tested and finalised	Field Testing of TB Audit tool	On track for completion for Q1
Development of STI Data Quality Audit tool	STI Audit tool developed	STI data quality audit tool pre-tested and finalised	Field Testing of STI Audit tool	On track for completion for Q1
Supportive Supervision Visits	2 Supportive Supervision Visit reports	Supportive Supervision Visits were conducted as per schedule, and 80% of data quality gaps were addressed	2 Approved Supportive Supervision visit reports submitted	On track for completion for Q1
Updated/Integrated Treatment and Prevention Databases	Final report on the updating and integration of the Treatment and Prevention Databases	Updated/Integrated treatment and prevention databases operationalized	N/A	
Database training for TSIS and DHIS 2	Database training for TSIS and DHIS 2	Database training for DHIS 2 conducted by Q2	N/A	
PLHIV estimate	PLHIV estimate	PLHIV (Dec 2025) estimated by Q1	PLHIV (Dec 2025) estimated	On track for completion for Q1
Hotspot mapping for TB	Hotspot mapping for TB	Hotspot map showing TB cases overlaid with risk factors	N/A	

Major Initiatives/ Activities	Key Outputs	Performance Measures/Indicators	Targets (Projections)	
			April - June	Status
HIV/STI/TB Data Triangulation Meeting	HIV/STI/TB Data Triangulation Meeting	80% of data triangulated	N/A	
Quarterly Performance Review	Quarterly Performance Review	Report from the 1 national review meetings and 8 regional/CSO meetings for the year.	N/A	
M&E Technical Working Group Meeting	M&E Technical Working Group Meeting	2 action sheet updates from M&E TWG meetings	M&E meeting held with key stakeholders	On track for completion for Q1
TSIS Quarterly Data Review Meeting	TSIS Quarterly Data Review Meeting	80% of actions completed on plans	TSIS Quarterly Data Review Meeting and approved report	On track for completion for Q1
Quarterly HIV, STI, TB Database Bulletins	Quarterly HIV, STI, TB Database Bulletins	3 HIV/STI/TB Database Bulletins	N/A	
Assessment of Prevention Data Quality Audit	Approved Prevention Data Quality Audit reports	Prevention DQA conducted with at least 60% of corrective actions implemented	Prevention Data Quality Audits (SERHA, NERHA,WRHA,SRHA) Prevention DQA report and action plan (SERHA,SRHA NERHA)	On track for completion for Q1